Boston Children's Hospital
Until every child is well™

HARVARD MEDICAL SCHOOL
TEACHING HOSPITAL

Office of General Counsel
300 Longwood Avenue, BCH3046
Boston, Massachusetts 02115
617-355-6800 | 617-355-6108 | *Fax* 617-730-1952

March 21, 2024

VIA EMAIL

William Cooney, Investigator
william.cooney2@mass.gov
Carol Murchison, Administrative Assistant
Carol.murchison@mass.gov
Massachusetts Commission Against Discrimination
1 Ashburton Place
Boston, MA 02108
bospositionstmts@mass.gov

Re: ***Alan Turner v. Waltham Children's Hospital***
MCAD No. 23-NEM-03448
EEOC No. 16C-2024-00548

Dear Mr. Cooney and Ms. Murchison:

I hope this letter finds you well. In connection with the above-entitled matter, enclosed please find the following pleading:

- Position Statement of Respondent, Children's Waltham Medical Center, LLC.

Please feel free to contact me should you have any questions regarding the enclosed. Thank you.

Sincerely,

James J. Horgan
Deputy General Counsel

JJH/hao
Enclosures

cc: Mr. Alan Turner, via first-class mail w/ encl.

COMMONWEALTH OF MASSACHUSETTS

ALAN TURNER,	)	
	)	
Complainant	)	
	)	**MCAD NO. 23-NEM-03884**
v.	)	
	)	**EEOC NO. 16C-2024-00548**
WALTHAM CHILDREN'S HOSPITAL[1],	)	
	)	
Respondent	)	
	)	

POSITION STATEMENT OF RESPONDENT
BOSTON CHILDREN'S HOSPITAL

I. INTRODUCTION

Boston Children's Hospital ("BCH", the "Hospital" or "Respondent") is a charitable corporation primarily engaged as an academic medical center focused on medical research and the medical care of children. BCH is a full-service medical center open 24 hours a day, every day. Because BCH treats some of the most medically complex children in the world, patient care plays an extremely important role at BCH. The Boston Children's Hospital Inpatient Psychiatry Service ("IPS") provides critical mental health services to vulnerable children. IPS has two locations, at BCH's Main Campus on Longwood Avenue, Boston, MA and at BCH's Waltham campus on Hope Avenue in Waltham, Massachusetts ("IPS-Waltham"). Mr. Turner's ("Mr. Turner" or "Complainant") allegations arise at IPS – Waltham. Mr. Turner's claims of race-based discrimination are without merit. As set forth in more detail below:

- Mr. Turner was employed for less than 5 months as a Behavioral Health Counselor at Boston Children's Hospital Inpatient Psychiatry Service (IPS) – Waltham.
- During Mr. Turner's tenure, this job title was 69% non-white and 38% black, demonstrating that this job title is very diverse.
- During his brief tenure, he was found to have sexually harassed multiple female employees of IPS Waltham and was found to have significant boundary challenges with teenage female patients at IPS.
- The allegations against Mr. Turner were substantiated by the Hospital's Human Resources Department.

[1] The Respondent is misnamed, the correct name is Boston Children's Hospital, although that location is in Waltham.

- Moreover, in the course of the interviews of Mr. Turner he acknowledged that he had been investigated for sexual harassment at previous employers and expressed little understanding of how to conform his behavior to the Hospital policy.

Accordingly, Mr. Turner was terminated for repeated substantiated violations of the Hospital Sexual Harassment Policy.

II. BACKGROUND

A. Boston Children's Hospital Inpatient Psychiatry Service (IPS) – Waltham

Boston Children's Hospital Inpatient Psychiatry Service (IPS) - Waltham provides inpatient mental health services to children and adolescents in an acute, locked mental health care setting. The patients treated in the IPS-Waltham are some of the most vulnerable patients cared for by BCH. At Waltham, the 12-bed, secure inpatient psychiatric unit is located in the west wing of the main building, known as 5 West – Waltham – IPS. IPS treats children and adolescents ages 8 to 17 suffering from debilitating psychiatric conditions. IPS provides patient and family centered care. The clinicians use focused psychiatric assessment and current, effective treatments within a safe environment to help stabilize a patient's behavioral health crisis.

Given how vulnerable these patients are, it is necessary that clinicians maintain a high-level of professionalism in IPS because of the severe psychiatric disorders that patients are receiving treatment for. The goal is to help empower the patient and family to make changes that will make a difference in terms of safety at home, school, and in the community. IPS clinicians work together with the patient and family to determine the next best steps for transitioning out of the acute-care setting of the IPS.

B. Mr. Turner's Employment Within the Inpatient Psychiatry Service (IPS) – Waltham

Mr. Turner was hired on July 17, 2023, as a Behavioral Health Counselor at Boston Children's Hospital Inpatient Psychiatry Service (IPS) - Waltham. He was hired part time, 36 hours per week working 8-hour shifts, day/evening rotation, occasional nights, every other weekend, and rotating holidays. *See,* **Complainant's Offer Letter, attached hereto Exhibit A** ("Offer Letter"). When Mr. Turner was hired, he was one of six Black Behavioral Health Counselors, the majority racial demographic that comprised this job title in IPS – Waltham. Overall this job title was 38% black and 69% non-white.

As a Behavioral Health Counselor, Mr. Turner formally reported to Nurse Manager Sharon Arnow ("Ms. Arnow"). A Boston Children's Hospital Behavioral Health Counselor is expected to provide highly-skilled therapeutic services and safety management of patients hospitalized with psychiatric illnesses. Behavior Health Counselors spend a significant amount of time with patients. This position also supports the multidisciplinary team to plan, provide and communicate patient treatment/behavior goals and requirements. *See* **Behavioral Health Counselor job description, attached hereto as Exhibit B** ("Job Description").

C. Mr. Turner Exhibited Potential Boundary Concerns with Female Patients

From the beginning of his tenure at BCH, Mr. Turner's colleagues observed that he would inappropriately interact with female patients. As a Behavioral Health Counselor, Mr. Turner's role was to engage with patients in a strictly professional manner in order to watch for any safety concerns. Part of this job responsibility also involved maintaining a physically safe environment for the patients. **Job Description at 1-2**. As a Behavioral Health Counselor, Mr. Turner was required to maintain careful boundaries with these young patients.

Many of IPS' patients suffer from severe psychiatric illnesses and are prone to a variety of attachment disorders and other conditions which make them vulnerable to ("RAD") if forming unhealthy emotional bonds with their medical caretakers because of mental health struggles, emotional neglect and/or abuse at a young age.

Mr. Turner disregarded these clinically important boundaries by spending hours talking to adolescent female patients and broaching highly inappropriate topics. In one conversation with a patient, Mr. Turner told the patient that he wants to "take her out" when she gets to college. Mr. Turner knew this female patient from a prior, brief role he had at a different psychiatric institution. On another occasion, Mr. Turner said he wanted to "party" with patients when they go to college. On numerous occasions, patients told IPS staff that they only wanted to interact with Mr. Turner, a major red flag within a highly secure, enclosed treatment center. This resulted in multiple efforts to counsel Mr. Turner to be mindful of boundaries with the patients.

During a meeting about Mr. Turner's sexual harassment of co-workers, Complainant admitted to Lead Employee Relations Advisor, Scott Shannon ("Mr. Shannon"), that he did not understand why recurrent conversations concerning patient boundaries "keep happening to me." In this conversation, Mr. Turner admitted that he was accused of inappropriately touching a young patient in his previous role at Carney Hospital. Mr. Turner did not take accountability for this incident, nor any of the many incidents of boundary concerns with patients in IPS. A review of Mr. Turner's resume shows a repeated pattern of very short employment periods at a variety of mental health settings.

D. Mr. Turner Repeatedly Violated the Hospital Sexual Harassment Policy

Mr. Turner committed numerous violations of Hospital policy against sexual harassment. *See*, **BCH Policy Against Sexual Harassment, attached as Exhibit C at 1-2** ("Sexual Harassment Policy"). This policy applies to all BCH employees, and BCH is strongly committed to providing a workplace that is free of harassment. BCH's Code of Conduct sets forth additional rules and expectations with which all employees are expected to comply. Specifically, the Code of Conduct makes clear that communication, respect, excellence, accountability, teamwork, and innovation are core values of the organization. *See,* **Code of Conduct, attached hereto Exhibit D** ("Code of Conduct").

Ms. Arnow became aware of Mr. Turner's inappropriate interactions with female workers on November 9, 2023. The reporting individual was a staff nurse in ISP who had worked with Mr. Turner during a night shift ("Victim One") During her shift, this nurse was cornered by Mr. Turner

in a kitchenette and interrogated about her dating life, a conversation that Mr. Turner even admits he initiated in his MCAD Charge. **Mr. Turner asked if Victim One dated outside her race. When she asked if he was suggesting that she should date him**, Mr. Turner looked this nurse up and down and repeatedly suggested that they "should get together." He also said words to the effect "who said anything about dating" suggesting to Victim One that he was suggesting a purely sexual relationship.

While reporting this incident to Ms. Arnow, Victim One also reported that Mr. Turner had engaged in similar concerning interactions with two other female colleagues. These interactions included unwanted touching, unsolicited hugs, and comments regarding the physical appearance of a colleague and how "the gym was treating a [her] right" Or words to that effect.

Alarmed by these reports, Ms. Arnow escalated the situation to the Human Resources Department and the matter was handled by Employee Relations Specialist, Scott Shannon. Because of the seriousness of these concerns, Mr. Shannon put Mr. Turner on paid administrative leave, on November 10, 2023, until the investigation was complete.

Next, Mr. Shannon met with each victim to gather information about their interactions with Mr. Turner. Victim One repeated the same, concerning interaction with Mr. Turner that she reported to Ms. Arnow. Victim One's report was extremely consistent, she indicated that Mr. Turner had issues respecting boundaries with staff as well as patients. She stated that he had repeatedly approached her, but focused on the most recent event where Mr. Turner had inquired about her dating life and whether she ever dates outside her race, looked her up and down in an inappropriate manner, and said "if you are down you should hit me up" or words to that effect. When Victim One stated that she does not "date people [she] works with", that is when Mr. Turner was clear about his intent for a sexual relationship when he responded "who said anything about dating?" Again, Victim One reasonably interpreted this as an invitation to a purely sexual relationship. Victim One made clear that the conduct was unwelcome and made her feel uncomfortable. She also informed HR that she had observed Mr. Turner socializing in the hallways with the patients and found that he was not observing professional boundaries with the patients. Victim One also identified two other staff who could speak to their own interactions with Mr. Turner which were also concerning and suggested that this behavior was part of a pattern for Mr. Turner.

Victim Two, a behavioral health technician in ISP, told Mr. Shannon that Mr. Turner repeatedly commented on her body and said things to the effect her body "looks really good" and "the gym has been treating you quite well". He also would inquire about her dating relationships. In one particular incident, Victim Two described how Mr. Turner put his hand on the back of her chair, leaned in and made inappropriate comments about how he would "treat her if she was with him." Again, Victim Two interpreted this as an invitation to a sexual relationship with Mr. Turner. This behavior was unwelcome and made Victim Two uncomfortable. Victim 2 also added that she had heard Mr. Turner failing to observe proper boundaries with teenage patients, including directly observing a conversation where he said that he would "party" with them when they get to college

Next Mr. Shannon interviewed Reporter 3, a staff nurse. Reporter 3 primarily provided information regarding Mr. Turner's failure to observe professional boundaries with patients. She

repeatedly observed him socializing with patients. In a particularly disturbing interaction, Mr. Turner was observed to say to teenage patients that he would be "into" "getting together" and "partying" with the patients when they are older. The boundary violations were regular and Reporter 3 tried to counsel him to be more attentive to boundaries which Mr. Turner never did.

After interviewing the three victims, Mr. Shannon met with Complainant to discuss the allegations. Mr. Turner made some revealing admissions during the interview. First, he stated that "he had been accused of touching a young patient at another institution where he worked. Mr. Turner also admitted that he was terminated from that position. When asked why people would make these allegations, Mr. Turner posited that they are "jealous of his relationship with the kids". This again shows that he does not understand the nature of the professional role he holds. He is not supposed to develop a "relationship" with the kids, these are patients toward whom he has a professional duty.

As to the sexual harassment, Mr. Turner essentially stated he "was just being himself" and was "offering advice" when he interacted with these colleagues. Moreover, he did not show any understanding of why these interactions made staff uncomfortable or show any remorse or regret that his behavior was unwelcome or had impacted his colleagues negatively. *See*, **Decision to Terminate, attached hereto as Exhibit E.**

Based on this information Mr. Shannon substantiated the allegations and found them to be a direct violation of BCH's Sexual Harassment Policy. After a thorough investigation substantiating Mr. Turner's violations of the Sexual Harassment Policy, Human Resources in consultation with Mr. Turner's management, BCH terminated Complainant's employment on December 7, 2023.

E. Mr. Turner's Allegations of Incorrect Pay[2]

Mr. Turner incorrectly alleges that BCH did not provide his entire paycheck on December 8, 2023. On occasions where IPS has a very hard to fill shift they will offer incentive pay opportunities in the form of increased pay rates for those shifts. Those opportunities are sent to all eligible employees by email. Mr. Turner wrongfully alleges that two of the shifts he worked on October 20th and 21st, 2023, were incentive pay shifts. The Hospital did not provide incentive pay opportunities on either of these dates. Accordingly, Mr. Turner was appropriately compensated for his time.

III. Complaint's Allegations

The specific allegations contained in the Charge, as well as Respondent's response to each of these allegations, are set out below.

[2] Respondent submits that allegations of Wage violations are not properly before the MCAD, but Respondent provides a narrative explanation to reassure the Commission that there is no merit to the allegation to the extent it is being considered as a discriminatory act.

Allegation No. 1

I identify my race as Black.

Response No. 1

Respondent admits that Complainant self-identified as Black/African American in Respondent's internal Human Resources systems.

Allegation No. 2

In July 2023, Respondent hired me as a Mental Health Counselor. I was one of four Black men who worked in my Department and I felt others not in our protected class were treated better than us. For example, Respondent would give more leniency to White coworkers.

Response No. 2

Respondent denies the allegations as drafted. On July 17, 2023, Mr. Turner was hired as a Behavioral Health Counselor in the Inpatient Psychiatry Unit at BCH Waltham.

Further answering, based on Respondent's internal Human Resources systems, when Mr. Turner was hired he was one of six Black Behavioral Health Counselors in IPS-Waltham. At the time Mr. Turner was hired, this position in Waltham was 38% Black/African American, and 69% ***non***-white. This clearly creates an inference that there is no racial bias against Black employees in the department. Moreover, Complainant provides no evidence of comparators to support his assertion that similarly situated white employees are treated more favorably in situations of sexual harassment and professional boundary issues. Respondent denies the remaining portions of the allegation.

Allegation No. 3

In July 2023, when I started working for Respondent, the kids I worked with grew to me with how I worked with them. Respondent loved me, they praised me for how well I worked with the kids. As time went on, coworkers such as Nurse Kaylin LNU (White), Nurse Chloe LNU (White), Administrative Assistance Brittany LNU (White), and Nurse Lauren LNU (White) started giving me the cold shoulder and stopped speaking to me.

Response No. 3

Respondent denies that Mr. Turner appropriately performed his job duties as he was spoken to about his failure to observe professional boundaries with patients. Further answering, Respondent denies the adequacy of Mr. Turner's work performance because of his sexual harassment of fellow employees. To the extent staff were not interested in engaging with Mr. Turner is consistent with having experienced sexual harassment by Mr. Turner and is not

suggestive of any discriminatory animus. Respondent denies the remaining portions of Allegation No. 3.

Allegation No. 4

On November 9, 2023, I received a text message from my supervisor Sharon LNU (White) telling me she needed to speak with me. I never ended up speaking with Sharon.

Response No. 4

Respondent admits that Mr. Turner's supervisor, Nurse Manager Sharon Arnow, tried to schedule a meeting to speak with Mr. Turner. The meeting never took place because Mr. Turner failed to respond. Respondent denies the remaining portions of Allegation No. 4.

Allegation No. 5

On November 10, 2023, I received an email from Lead Employee Relations Advisor Scott Shannon (White) that Respondent was placing me on paid administrative leave pending investigation of accusations against me. Shannon told me that he could not tell me what I was being accused of or who was making the accusations.

Response No. 5

Respondent admits that Mr. Turner was placed on paid administrative leave due to the seriousness of the sexual harassment allegations made against him. Due to concerns for retaliation Mr. Shannon would not have revealed the names of the accusers. Respondent denies the remaining portions of Allegation No. 5.

Allegation No. 6

In or around mid-November 11, 2023, Shannon called me to tell me that two more coworkers came forward and harassed them. I asked him what I allegedly did. One person said I put my hand on her lower back. I told him I used to teach about boundaries when I worked as a Supervisor for nine years, I know my boundaries and told him that I did not do it. The only person I ever came in contact with was a coworker I had trained, Kelly LNU (White). She was the only person I ever really spoke with and had a good relationship with at Respondent. I told him I know who I hugged and based on our relationship; it would not be her making these accusations. Shannon also told me someone said that I allegedly had inappropriate conversations with a coworker. These were all untrue. Shannon informed me that Lauren was one of my accusers. I never had any inappropriate conversations with her and that I tried asking her out. I told Shannon that I had only ever asked her if she ever thought about dating outside of her Race because she was having issues dating White men. She said no and that, "once you go black, you never go back." I asked her what she meant by that and that I never said "Black," just "outside her race." I told her that I could take her out and find her a man, no strings attached. Shannon told me that my accusers said that I looked at them funny and was looking them up and down.

Response No. 6

Respondent admits that the Complainant spoke with the Respondent's Human Resources

Department about multiple concerning interactions with female colleagues. Further answering, Respondent states that Mr. Turner initiated a conversation specifically relating to dating outside your race and interrogating her about her dating life and preferences. Moreover he attempted to initiate a sexual relationship with his coworker. In that context of these highly intrusive comments, may have responded, "are you saying" … "once you go Black, you never go back?" Respondent denies the remaining allegations.

Allegation No. 7

On November 30, 2023, Sharon sent me a text message and asked to speak with me. We set up a time to talk at 1:00 PM. I forgot about a family matter I had at that time. I texted her asking to meet the following Monday at 1:00 PM instead. She then texted me back that HR would be reaching out.

Response No. 7

Respondent denies the allegations as drafted. Respondent admits that Complainant's supervisor, Nurse Manager Sharon Arnow, sent him a text message on December 7, 2023, to schedule a meeting to speak with him at 1:00pm. This meeting never occurred because Respondent did not show up. Ms. Arnow then sent a subsequent text message that Human Resources would be in communication with Complainant. Respondent denies the remaining portions of Allegation No. 7.

Allegation No. 8

On December 7, 2023, Shannon emailed me to tell me that Leadership was terminating my employment effective immediately because I did not speak with Leadership when I did not make my meeting with Sharon. I told him that I had legitimate reasons and that I had contacted Sharon ahead of time to inform her that I needed to postpone our meeting because I forgot I had to deal with a family matter on November 30, 2023 at 1:00 PM and asked to reschedule. He never responded to my email.

Response No. 8

Respondent denies the allegations as drafted. Respondent admits that the Complainant was terminated on December 7, 2023 due to his violations of BCH's Discrimination & Sexual Harassment policy. Respondent denies the remaining portions of Allegation No. 8.

Allegation No. 9

On December 8, 2023, Respondent did not give me my entire paycheck. It had about $1,200 missing.

Response No. 9

Respondent denies Allegation No. 9.

IV. ANALYSIS

As demonstrated by the recitation of the facts above, Mr. Turner's claim of racial discrimination is without merit.

A. Mr. Turner's Racial Discrimination Claim is Without Merit

1. Mr. Turner cannot satisfy his prima facie case.

Mr. Turner's claims of race-based termination discrimination are without factual or legal merit. "[T]he central focus of inquiry [in a discrimination case] is whether the employer penalizes" an employee "because of *their race,* color, religion, sex, or national origin." *Harvard Law Sch. Coal. For Civil Rights v. President & Fellows of Harvard. Coll.*, 413 Mass. 66, 69 (1992) (emphasis added). To establish a *prima facie* case if racial and/or color discrimination, Mr. Turner must show that "(1) he is a member of a protected class; (2) she was qualified for the job; (3) the [Hospital] took an adverse employment action against [him]; and (4) the position remained open or was filled by a person with similar qualifications." *Kosereis v. Rhode Island*, 331 F.3d 207, 212-13 (1st Cir. 2003). Here, Mr. Turner cannot satisfy his *prima facie* burden and show that he was qualified for the job (prong two) or that the position was filled by a person with similar qualifications (prong four).

First, Mr. Turner has not demonstrated that he was qualified for his job. Mr. Turner inappropriately engaged with female patients and was ultimately terminated for his multiple incidents of sexual harassment of his female colleagues. Care professionals working in IPS are required to maintain careful boundaries while interacting with patients suffering from a multitude of psychiatric conditions. If these boundaries are ignored, a patient's treatment can regress thwarting IPS' goals of transitioning patients back into their homes, schools, and communities. Mr. Turner took advantage of vulnerable female patients by making "exceptions" under these high standards of professionalism and inappropriately conversing about personal topics. Mr. Turner's actions resulted in patients rejecting necessary treatment from other care professionals for their psychiatric conditions, undermining IPS' overall clinical objectives. Despite warnings about his behavior, Mr. Turner continued to inappropriately engage with female patients.

Mr. Turner also demonstrated he was unqualified for his job because of his sexual harassment of female colleagues. Boston Children's Hospital, like the Massachusetts Commission Against Discrimination (MCAD), strictly enforces state laws that prohibit sexual harassment in the workplace. *See,* **MCAD Guidelines on 151B, attached hereto EXHIBIT F** ("MCAD Guidelines'); **Sexual Harassment Policy.** As his offer letter states, Mr. Turner's employment was contingent on his compliance with the Hospital's policies and standards of conduct, including Respondent's Discrimination and Sexual Harassment policy. **Offer Letter at 2.** During his tenure at Boston Children's, Respondent's Human Resources Department determined that Mr. Turner inappropriately conversed, touched, and interacted with several different female colleagues. One female colleague was so alarmed by Mr. Turner's advances that she reported her interaction to management, who swiftly reported the incident, along with others, to Human Resources. Taken

together, Mr. Turner's behaviors were a clear violation of hospital policy and a major disruption to the important care these employees were delivering to IPS patients.

Mr. Turner has not fulfilled his prima facie duty under prong four by showing that the Hospital planned to replace him with a non-Black person, or that the part-time Behavior Health Counselor remained open and BCH sought to fill it. In fact, when Mr. Turner was put on administrative leave, BCH filled his position with a Black Behavioral Health Counselor on November, 20, 2023. Mr. Turner has not satisfied his prima facie burden, and his discrimination claim should be dismissed because he has not identified *any* comparator or potential comparator regarding his position.

2. The Hospital has articulated a legitimate, non-discriminatory reason for its employment decisions.

Even assuming Mr. Turner has satisfied his *prima facie* burden for her discrimination claim (which he has not), this claim still fails because the Hospital has articulated a legitimate, non-discriminatory reason for its employment decision. *See Blare v. Husky Injection Molding Sys. Bos., Inc.,* 419 Mass. 437, 441 (1995) ("the employer can rebut the presumption created by the prima facie case by articulating a legitimate, nondiscriminatory reason for its hiring decision"). The Hospital has clearly articulated its reasons for Mr. Turner's administrative leave and termination: Mr. Turner engaged in a series of unprofessional and inappropriate sexual harassment incidents towards his female colleagues, which precipitated his employment termination. **Decision to Terminate**.

At this stage in the burden shifting analysis, Mr. Turner has also offered no explanation regarding how his termination is casually related to his membership in any protected class. Mr. Turner circularly claims that non-Black employees received "leniency," but offers no comparator evidence to support this assertion. Namely, that other non-Black employees repeatedly sexually harassed colleagues and were not terminated for their misconduct. As previously stated, Boston Children's has a policy against sexual harassment and applies it without regard to race.

Mr. Turner's claims are particularly misplaced in the broader culture of Boston Children's Hospital and the specific context of the Waltham IPS. Boston Children's Hospital remains committed to the principles of health equity, diversity, inclusion, and the promotion of anti-racist principles. *See,* **"About Boston Children's Hospital Website", attached hereto as Exhibit G** ("About BCH") (recognizing that the Hospital is "committed to working together to support health equity and promote anti-racist practices"). In fact, in August 2020, the Hospital announced "Boston Children's Declaration on Equity, Diversity, and Inclusivity," which includes a commitment to equity, diversity, and various anti-racist practices. *Se,e* **"Boston Children's Declaration on Equity, Diversity, and Inclusivity," attached hereto as Exhibit H** ("BCH Declaration")**.** Mr. Turner's race-based discrimination allegation is particularly baseless in the context of the demographic composition of the Behavioral Health Counselors in IPS-Waltham. Of the approximately 21 Behavioral Health Counselors at the time of Mr. Turner's administrative leave, 38% self-identified as black, and 69% are non-white. Mr. Turner's assertion that he was discriminated against by leadership of his area due to race is unsupported. Mr. Turner's claims are not just insufficient as a matter of fact and law, they are also unsupported as a matter of common

sense. The Charge must be dismissed.

Ultimately, Mr. Turner does not offer sufficient evidence to establish that the Hospital took an adverse employment action taken against him *because* of his race. The uncontroverted evidence demonstrates that the cause for Mr. Turner's termination was wholly unrelated to race; the Hospital terminated Mr. Turner's employment after substantiating multiple, serious claims of sexual harassment against him, which clearly violate Respondent's anti-discrimination policies. Mr. Turner's discrimination claim must be dismissed.

V. AFFIRMATIVE DEFENSES

Unless otherwise specified herein, Respondent denies, or lacks sufficient information to answer, each and every allegation in the Charge.

Respondent asserts the following defenses in response to the allegations contained in the Charge:

1. Complainant failed to establish a *prima facie* case with respect to his claims of discrimination.

2. Complainant was unable to file a clear and concise statement of alleged discriminatory acts that would allow the Respondent to sufficiently respond to the allegations.

3. Complainant failed to state a claim upon which relief may be granted.

4. Complaint's allegations concerning matters that occurred prior to the applicable statute of limitations are barred by the statute of limitations.

5. Every action taken by Respondent with respect to Complainant's employment was taken for a legitimate business purpose and was consistent with the law.

6. The Hospital is a charitable corporation and therefore entitled to charitable immunity on some or all of those claims.

Respondent reserves its right to raise additional defenses, which may become apparent during the course of the Commission's proceedings.

VI. CONCLUSION

Complainant has presented no evidence of his claims of discrimination. Because Complainant cannot sustain any of his claims, Boston Children's Hospital respectfully requests that the Charge be dismissed for lack of probable cause.

I, upon information and belief, state under the pains and penalties of perjury, that I am authorized to verify this position statement on behalf of the Respondent, Boston Children's Hospital, and that while I do not have personal knowledge of all of the various facts recited in these answers, the response was prepared with the assistance and advice of counsel and information from the Respondent, upon which I have relied; and that the facts stated, subject to these limitations are true to the best of my knowledge, information and belief.

For the Respondent:

March 21, 2024
Date

Sharon Arnow
Sharon Arnow, RN
Nurse Manager
Boston Children's Hospital
300 Longwood Avenue, Boston, MA 02115

Legal Counsel:

March 21, 2024
Date

James J. Horgan
James J. Horgan, Esq.
Deputy General Counsel
Office of the General Counsel
Boston Children's Hospital
300 Longwood Avenue, BCH 3046
Boston, MA 02115
Phone: (617) 355-6108
Fax: (617) 730-1952

CERTIFICATE OF SERVICE

I, James J. Horgan, Esq., attorney for Boston Children's Hospital, hereby certify that on March 21, 2024 I caused notice to be given of the within Position Statement as follows:

via e-mail, to the following interested parties:

Carol Murchison, Administrative Assistant
Massachusetts Commission Against Discrimination
1 Ashburton Place
Boston, MA 02108

William Cooney, Investigator
Massachusetts Commission Against Discrimination
1 Ashburton Place
Boston, MA 02108

via first-class mail, to the following interested party:

Alan Turner
30 Arthur Street
Quincy, MA 02169

James J. Horgan

James J. Horgan, Esq.

EXHIBIT A

Nurse Recruitment
Department of Nursing/Patient Services
300 Longwood Avenue, LM6230, Boston, Massachusetts 02115
617-355-6000 | bostonchildrens.org

June 23, 2023

Alan Turner
770 Cummings Highway
Apt 15
Boston, MA 02126

Dear Alan,

On behalf of the entire team, we would like to welcome you to Boston Children's Hospital. Our mission is to provide the highest quality health care, be the leading source of research and discovery, to educate the next generation of leaders in child health and to enhance the health and well-being of the children and families in our local community.

We are delighted that you have accepted our offer for the Behavioral Health Counselor position in the Inpatient Psychiatry Waltham in Patient Services-Nursing. The following provides further detail on your position.

Description of the Position:
The position is part time, 36 hours per week and you will be working 8-hour shifts, day/evening rotation, occasional nights, every other weekend, and rotating holidays. As discussed, your start date will be July 17, 2023 and you will be reporting to Sharon Arnow MSN, RN, CNL, Nurse Manager. **Please note that in the event of a public health emergency you may be asked to assume additional duties in other areas of the organization.**

Compensation and Benefits:
Your salary will be $29.38 hourly and you will be paid bi-weekly.Your position is considered non-exempt and has a three month review period which begins on your start date.This position is benefits eligible. You may also be eligible for premium compensation that is offered to Behavioral Health Counselors, such as shift and overtime premiums, as applicable.

Conditions of Employment:
Employment with Boston Children's Hospital is contingent upon a satisfactory reference check, a comprehensive background, including but not limited to criminal history check, verification of licensing, credentials and education (if required for the position), completion of a pre-employment fitness for duty assessment with Occupational Health Services, including required immunizations, execution of a Participation Agreement and proof of your eligibility to work in the United States. You will receive an email from HROnboarding@childrens.harvard.edu directing you to our online portal where you will access more information and complete required forms. During your orientation you will take a written test and complete a skills checklist. You must receive a minimum score of 80% on the written test and successfully complete the skills checklist.

Employment at Boston Children's Hospital is at will. This means that your employment is not for a guaranteed period of time, and either you or the Hospital may terminate the employment relationship at any time and for any reason. Your employment is also subject to meeting performance expectations, compliance with the Hospital's policies and standards of conduct, infection control requirements and vaccination against COVID-19 and Flu (unless you are eligible for a medical or religious exemption).

We look forward to having you as a member of the Boston Children's team and hope that your new position will be a valuable experience for you. If you have any further questions, please feel free to contact me at 617-355-5009.

Sincerely,

Olivia Mathews

Olivia Mathews
Talent Acquisition Ambassador

cc: Sharon Arnow MSN, RN, CNL, Nurse Manager

EXHIBIT B

Behavioral Health Counselor

Job Code	FLSA Status	Job Grade
6150	Nonexempt	300

Recruiting Summary

General Summary

Under general supervision and following established procedures, methods, and protocols, provides skilled therapeutic services and safety management of patients hospitalized with psychiatric illnesses. Supports the multidisciplinary team to plan, provide and communicate patient treatment/behavior goals and requirements.

Values Statement

Embodies the Hospital's core values in all interactions with patients, families, and colleagues.

Duties Notice

The statements below describe the essential duties of the person or persons assigned to this job. They are not intended as an exhaustive list of all job duties and responsibilities. In the event of a public health emergency, you may be asked to assume additional duties in other areas of the organization if needed.

Principal Duties and Responsibilities

1. Monitors and maintains safety of patients in accordance with established procedures and protocols.
2. Maintains a physically safe environment. Checks and screens items brought into unit. Assists Registered Nurse to structure and modulate the level of stimulation in the unit. Conducts patient

and environmental checks and identifies risk. Reports identified risk to Registered Nurse.
3. Supports and assists patients with routine activities such as bathing, dressing, and eating.
4. Identifies and reports changes in behavior that represent unsafe situations. Respectful re-direction of unsafe behavior and limit setting as needed.
5. Provides skilled therapeutic care, incorporating and applying established behavior management protocols as needed to meet treatment goals and objectives.
6. Assists in arranging and conducting group programs and activities.
7. Develops and maintains supportive relationships with patients and families. Identifies specific needs of patients and families and incorporates into patient care as appropriate.
8. May collect, measure and record basic patient data, such as vital signs, intake, and output, and obtains specimens according to established procedures and schedules.
9. Records medical data and care provided in patient medical record in accordance with established documentation standards.
10. Serves as member of multidisciplinary team for assigned patients.
11. Assists in planning and implementing individual treatment plans and communicates patient status, observations, and recommendations.
12. Participates in patient care rounds and team meetings.
13. Develops and maintains knowledge of current occupational practices and techniques through participation in professional organizations, attendance at workshops, in-service sessions, and other continuing education programs.

License

License Required	License Preferred
None Required	None Specified

Certification/Registration

Certification/Registration Required	Certification/Registration Preferred
1. Basic Life Support (BLS) at start date 2. De-escalation training (Crisis Prevention Institute, or other equivalent training) 3. Welle De-escalation training within one month of hire for new hires at this level	None Specified

Education

Required Education	Preferred Education
High School / GED	Associate's Degree

Area of Study

Required Area of Study	Preferred Area of Study

None Required	Psychology, Health Science, Human Services or related field preferred.

Work Experience

Required Work Experience	Preferred Work Experience
Internal: 1-2 Year of experience as a Behavioral Health Tech at BCH External: 1-2 Year of experience as a Behavioral Health, Mental Health Counselor or similar role in a similar setting Required: Practical experience in a behavioral health or educational setting required.	Practical experience in a Pediatric behavioral health or educational setting preferred.

Knowledge, Skills, and Abilities

1. Psychology: Knowledge of human behavior and performance; individual differences in ability, personality, and interests; learning and motivation; psychological research methods; and the assessment and treatment of behavioral and affective disorders.
2. Customer Service: Knowledge of principles and processes for providing exceptional customer/patient services.
3. English Language: Knowledge of the structure and content of the English language including the meaning and spelling of words, rules of composition, and grammar.
4. Active listening: Giving full attention to what other people are saying, taking time to understand the points being made, asking questions as appropriate, and not interrupting at inappropriate times.
5. Social Perceptiveness: Being aware of others' reactions and understanding why they react as they do.
6. Interpersonal Relationship: Fosters cooperation, collaboration, and communication to facilitate consensus and accomplish tasks.
7. Problem Solving: Work requires the skills to Identifying complex problems and reviewing related information to develop and evaluate options and implement solutions.
8. Critical Thinking: Using logic and reasoning to identify the strengths and weaknesses of alternative solutions, conclusions, or approaches to problems.
9. Persuasion: Persuading others to change their minds or behavior.
10. Instructing: Teaching others how to do something.
11. Reading Comprehension: Understanding written sentences and paragraphs in work-related documents.
12. Communication: Ability to delivers clear, effective communication via telephone, in written form, e-mail, or in person. Takes responsibility for understanding others, dealing effectively with conflicting views or issues and mediate fair solutions.
13. Teamwork: Ability to works with and helps others to accomplish objectives.
14. Attention to Detail: Ability to ensure information is complete and accurate; follows up with others to ensure that agreements and commitments have been fulfilled.
15. Oral Comprehension: The ability to listen to and understand information and ideas presented through spoken words and sentences.
16. Problem Sensitivity: The ability to tell when something is wrong or is likely to go wrong. It does not

involve solving the problem, only recognizing that there is a problem.

Physical Requirements

1. Frequent lifting and carrying patients/children and/or objects weighing up to 20 pounds and occasionally lifting and carrying patients/children and/or objects weighing 21 to 50 pounds.
2. Regular sitting, stooping and bending.
3. Regular reaching and grasping objects at, above and below shoulder level.
4. Regular grasping and fine manipulation with hands.
5. Regular proofreading and checking documents for accuracy.
6. Regular inputting/retrieving words or data into or from an automated/computer system.
7. Regularly pushing and/or pulling non-motorized equipment (e.g., stretcher, food cart, etc.) weighing 21 to over 50 pounds.

Bloodborne Pathogen Category

Potential Exposure to Bloodborne Pathogens. Job may require performance or tasks that involve potential for exposure to blood, body fluids, or tissues.

EXHIBIT C

Status Active PolicyStat ID 13735014

Effective 8/1/2022
Next Review 7/31/2025

Owner Carolyn Stetson
Department Human Resources
Applicability Boston Children's Hospital- Policies & Procedures
Chapter / Tags Human Resources (HR)

Where the world comes for answers

Policy Against Sexual Harassment

COPY

Internal Approval

VP, Human Resources

Scope

This policy applies to all Boston Children's Hospital (BCH) licensed locations, BCH operational and clinical departments, and staff (inclusive of W-2 employees, contracted staff, and members of the medical staff irrespective of their appointment category or employer), job applicants and volunteers. As applicable, the policy also applies to foundation practices leasing space at hospital-licensed locations.

Definitions

Sexual Harassment	Sexual advances, requests for sexual favors, and verbal or physical conduct of a sexual nature when: • submission to or rejection of such advances, requests or conduct is made either explicitly or implicity a term or condition of employment or as a basis for employment decisions; or • such advances, requests, or conduct have the purpose or effect of unreasonably interfering with an individual's work performance by creating an intimidating, hostile, humiliating or sexually offensive work environment. Under this definition, direct or implied requests by a supervisor for sexual favors in exchange for actual or promised job benefits such as favorable reviews, salary

	increases, promotions, increased benefits, or continued employment constitutes sexual harassment.

Policy Statements

- BCH prohibits any sexual harassment towards employees, staff, patients, families, applicants, vendors and/or visitors, made in person, online, or through any other platform, during and outside of work hours, within and away from the physical workplace.
- BCH prohibits retaliation against an individual who has complained about sexual harassment, assisted another person to report sexual harassment, filed a complaint of sexual harassment with BCH or a governmental agency, or participated in a sexual harassment investigation.
- This policy applies to any communication or behavior of a sexual nature towards employees, staff, patients, families, applicants, vendors and/or visitors, made in person, online, or through any other platform, during and outside of work hours, within and away from the physical workplace.

Procedures

- Sexual harassment can arise in a variety of circumstances and take different forms. While this is not an exhaustive list, some examples of prohibited conduct which may constitute sexual harassment include:
    - unwelcome sexual advances—whether they involve physical touching or not, including repeated or unwelcome flirtations or advances;
    - sexual epithets, jokes, written or verbal references to sexual conduct;
    - comments about an individual's sexual activity, deficiencies, or prowess or gossip concerning or inquiries into an individual’s sexual experiences;
    - displaying or communicating in any way, including through e-email, text messages, or social media, sexually suggestive objects, pictures, posters, cartoons or writings;
    - posting sexually offensive comments or graphic material on social media and blogs;
    - dissemination of sexually explicit voicemail, email, graphics, downloaded material or websites;
    - unwelcome leering, whistling, brushing against the body;
    - sexual gestures;
    - sexually suggestive, degrading or insulting comments;
    - inappropriate use of intimate or sexualized language; or
    - having a sexual relationship with a patient.
- Sexual harassment or inappropriate conduct directed at, concerning or affecting patients, families or visitors of BCH, as well as vendors and contractors, is prohibited.
- All individuals covered by this policy are expected to be aware of and comply with all ethical and professional rules of conduct applicable to their position, including any professional or regulatory rules or guidelines relating to inappropriate sexual relationships, harassment, or other inappropriate conduct.

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- Please note that communication or behaviors of a sexual nature or with a sexual theme can violate this policy even if such conduct would not meet the legal definition of sexual harassment. Conduct which is inappropriate, vulgar, offensive, intolerant or disrespectful of an individual can violate this policy, whether or not the conduct is unlawful.

Retaliation

- BCH prohibits retaliation against an individual who has complained about sexual harassment, assisted another individual to report sexual harassment, participated in an investigation into allegations of sexual harassment, or filed a charge with the EEOC or other state governmental agency. Retaliation includes any adverse employment action (e.g., demotion, assignment to less desirable projects, verbal abuse, etc.) taken against an individual because of the conduct described in this paragraph. All employees who experience or witness any conduct they believe to be retaliatory should immediately follow the reporting procedures stated below.
- Any individual who is found to have engaged in retaliation will be subject to disciplinary action, including termination of employment.

Filing Complaints of Sexual Harassment

- Individuals covered by this Policy who believe that they have experienced or witnessed sexual harassment at BCH, or retaliation should make a report to the BCH Human Resources Department immediately by emailing ***BCHEmployeeRelations@childrens.harvard.edu***. You may also use this email address to request a phone call or an in person or Zoom meeting with a member of the Employee Relations Department. You may also call (617) 355-7780 to speak with an Employee Service Center representive or an Employee Relations Advisor. For those who wish to visit Human Resources in person, we are located at 401 Park Drive, 6th Floor, Boston, MA.
- In addition, individuals who have experienced sexual harassment may also make a report to:
    - their direct supervisor;
    - another supervisor or manager within their own department;
    - the supervisor or manager of the department where the sexual harassment occurred if different from the individual's department;
    - Any director, vice-president, or officer of BCH;
    - Any Chief of Service of any clinical division or department; or
    - The Office of General Counsel.
- Any individual in a managerial or supervisory role at BCH who witnesses, receives a complaint, or becomes aware of sexual harassment by, affecting or involving a BCH Employee, member of the Medical Staff, Volunteer or Associated Personnel must report it to the Employee Relations Department in Human Resources immediately.
- Individuals who wish to make a complaint may do so by making an oral complaint and/or preparing a written statement. If the individual requires a translator in order to make the complaint, the Human Resources Department will assist with obtaining a translator.
- BCH is committed to investigating and resolving in a timely manner all complaints involving allegations of workplace sexual harassment. Human Resources may notify other entities and take other action if the conduct involves individuals who are not under BCH's control or

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direction.

- Individuals who have witnessed or experienced sexual harassment and are not employed by BCH should also contact their own employer's human resources department or designated representative for receiving complaints, in addition to the BCH Human Resources department.

Investigation

- An investigation into a complaint may include one or more of the following actions, as appropriate to the investigation:
    - Confidential interviews of the complainant(s) and witnesses.
    - Reviews of emails, internet activity and other computer information.
    - Individuals being placed on paid or unpaid administrative leave during the investigation.
    - Temporary changes in schedule or location as necessary
    - Requests not to contact
    - Involvement of the Office of General Counsel
    - Written findings
- BCH will maintain confidentiality to the extent practicable under the circumstances. The alleged subject of the complaint may learn of the complaint and the identity of the complainant as necessary. If the complainant has any concerns about retaliation, the complainant should notify Human Resources immediately.

Remedial Action

- BCH will take appropriate remedial action for violations of this Policy. Any individual who is found to have violated this policy will be subject to disciplinary action, up to and including termination of employment, or loss of privileges and termination of access to all BCH facilities.
- When appropriate, the results of the investigation and its disposition may be shared with the complainant and/or the subject of the complaint.

Filing a Complaint with the MCAD or EEOC

- Any individual who believes they have been subjected to sexual harassment may file an administrative complaint with either or both of the government agencies set forth below. Making a complaint under this policy does not prohibit the individual from filing a complaint with these agencies. Each of the agencies has a short time period for filing a claim (EEOC—300 days; MCAD—300 days).

Massachusetts Commission Against Discrimination (MCAD) One Ashburton Place, 6th Floor, Room 601 Boston, MA 02108 (617) 994-6000 TTY: (617) 994-6196	***U.S. Equal Employment Opportunity Commission (EEOC)*** JFK Federal Building, 475 Government Center Boston, MA 02203 (800) 669-4000 TTY: (800) 669-6820

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Related Content

- Disciplinary Policy and Standards of Conduct
- Diversity, Equal Employment Opportunity, and Affirmative Action Policy
- Policy Against Discrimination, Discriminatory Harassment and Retaliation
- Use of Social Media Policy
- NetLearning: Mandatory Yearly Review>Sexual Harassment and Discrimination

Approval Signatures

Step Description	Approver	Date
Site Admin	Ian Weijer: Site Admin	05/2023

Applicability

Boston Children's Hospital- Policies & Procedures

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EXHIBIT D

Status **Active** PolicyStat ID **12203809**

Effective	11/30/2021	Owner	Timothy Hogan
Next Review	11/29/2024	Department	Compliance
		Applicability	Boston Children's Hospital- Policies & Procedures
		Chapter / Tags	Anonymous Compliance Hotline, Code of Conduct, Communicating Clearly + 8 more

Where the world comes for answers

The Code of Conduct

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The Boston Children's community consists of people from many different backgrounds. Our patients, their families, our employees, and academic colleagues come from around the world. We embrace our diversity and do not discriminate based on religion, ethnicity, national origin, race, gender, gender orientation, age, handicap or veteran status. Children's will always be a community built around respect—respect for those we serve, and each other. We believe in the mission to improve global health

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for all children. We will continue to provide a safe and welcoming environment for all who share in that mission.

"At Boston Children's, we're committed to thinking differently, finding answers, and building a better tomorrow for children everywhere."

Introduction

Members of the Boston Children's Hospital Community are committed to honesty, integrity, and diligence in performing all daily activities and functions. Boston Children's adopted this Code of Conduct to affirm our noble principles, and to use them in setting the strategic and operational direction for the organization. Everyone involved with Boston Children's Hospital, whether you're an employee, a vendor, or students on a clinical rotation to maintain our high standards by reporting any potential violations to your supervisor, Office of the General Counsel (OGC), Compliance Department (857-218-4681), and/or the confidential

Anonymous Compliance Hotline at (888) 801-2805.
Interpreter services are available in 237 different languages

Calls are handled with appropriate confidentiality. Boston Children's Hospital does not engage in intimidation, retaliation or retribution against good faith reporters.

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Mission

Our mission is . . .
to provide the highest quality healthcare;
to be the leading source of research and discovery;
to educate the next generation of leaders in child health; and
to enhance the health and well-being of the children and families in our local community.

Our Goals

We have outlined five core goals that everyone should keep in mind so that we may accomplish our mission.

1. **Complex Patients**. Improve our capacity to care for all patients, particularly those with complex conditions.
2. **Growth and Accountable Care**. Develop relationships with other providers and payers that increase the number of patients with integrated access to complex care, and improve our ability to manage total medical expenses for defined populations and patients.

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3. **Research and Innovation Leadership**. Maintain our international leadership position in pediatric research and innovation to remain at the forefront of scientific and clinical advances, and to set us apart from other pediatric institutions.
4. **Operational and Administrative Effectiveness**. Improve administrative and operational systems and processes to support patients and providers and maximize their satisfaction.
5. **Staff and Employee Engagement**. Develop and execute programs and improved processes that increase the engagement, satisfaction, and capabilities of our staff and employees, as well as support and enhance our workforce diversity.

Our Values

Communication. Communicates clearly and responds thoughtfully and promptly.

Respect. Treats others with empathy and compassion; values our differences.

Excellence. Achieves the highest standards of performance every moment of every day.

Accountability. Hold each other to high standards of performance through attention to detail and personal integrity.

Teamwork. Collaborates to achieve cooperative solutions.

Innovation. Demonstrates and supports openness to change and new ideas.

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Commitment to Safety – High Reliability

We have an enterprise-wide commitment to becoming a highly reliable organization. This means doing things right the first time, every time. The three pillars of High Reliability are:

1. Speaking Up for Safety,
2. Communicating Clearly, and
3. Paying Attention to Detail

We are dedicated to increasing the quality of our alertness and attention to details. Failures are embraced in order to take action to stop further damage from occurring, to learn why an event occurred, and to prevent it from happening again. Our focus is on an increased sensitivity to operations – which means looking at what the organization is actually doing regardless of what we are supposed to do based on intentions, designs, and plans. These commitments strive to achieve ZERO preventable harms affecting any patient, family member, or team member at Boston Children's.

Scope

This Code applies to the Children's Hospital Corporation d/b/a Boston Children's Hospital ("Hospital"), its parent corporation, Children's Medical Center Corporation, and all of its corporate affiliates and satellite locations as well as the Hospital's affiliated medical foundations (collectively, "Hospital affiliates"), together with all of their respective trustees, directors, officers, employees, independent contractors, medical and research staff, faculty members, fellows, residents, interns, students, visiting faculty or

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scientists, consultants, and volunteers (whether compensated or not). This Code also applies to individuals and entities engaged by the Hospital to provide services on its behalf, including but not limited to professional consultants, contractors and vendors. All individuals are responsible for ensuring that they understand the Compliance Manual and policies and procedures that apply to their job role. Conduct that does not conform to the requirements of the Compliance Manual or applicable policies and procedures is deemed to be outside the scope of employment and will be subject to discipline.

Bylaws, Manuals, Policies and Procedures

The following topics are the major bylaws, manuals, and policies and procedures governing all individuals at Boston Children's. More specific individual department guides will assist you with your job responsibilities. Each section heading is a hyperlink to the referenced information.

Boston Children's Hospital Medical Staff Bylaws

The Medical Staff Bylaws govern medical appointments and practice at Boston Children's Hospital. Medical Staff need to demonstrate that their experience, educational background, training, professional competence, health, adherence to professional ethics, good reputation and ability to work with others are adequate to assure the Board of Trustees that they will perform their activities in a competent and ethical manner; and that any patient treated by them will receive efficient and quality care. Staff are required to meet the applicable qualifications, standards, and requirements set forth in the Bylaws and all organizational policies and procedures.

Clinical Services

Clinical Services' programs aim to be accessible, culturally appropriate, developmentally sensitive, socially responsible, and family-centered. The International Health Services Manual describes staff obligations and supports coordination of services for all international patients. The Patient Experience Manual is the sum of all interactions, shaped by our organization's culture, that influence patient perceptions across our continuum of care. The Ambulatory Services Manual describes departmental-specific information on operations, services, and includes policies and procedures.

Compliance

Our Compliance Program ensures that the organization conducts its business in compliance with all governing law; and that our operations are in accordance with established standards of business and professional ethics. The Board of Trustees charges the Chief Compliance Officer and Compliance Management Committee to review specific compliance matters that arise. The program has four key areas of focus: (1) Billing and Coding, (2) Conflict of Interest, (3) Privacy and Security, and (4) Research Compliance. Each area has specific policies and procedures that further describe important standard operating procedures. All individuals who work for or provide services to Boston Children's Hospital must also review the Compliance Manual – a broad overview of important organizational legal and ethical principles.

Equity, Diversity and Inclusion

"Being a leader in equity for all is an attainable goal, and with your support and your dedication, it's a goal

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we will keep working toward until it's a reality." Kevin B. Churchwell, MD, President and Chief Executive Officer

Boston Children's Hospital is committed to fostering a culture that honors diversity, respect, inclusion, and excellence, both within our system and across our community.

This requires a system of care that treats all patients and our workforce effectively and compassionately regardless of socioeconomic status, race, ethnicity, language, nationality, sexual orientation, gender identity, religion, geography, abilities, age, and life experience.

BCH is united around objectives to enrich our community and reduce healthcare disparities in pediatric care. We aim to advance culturally effective pediatric care through education and scholarship; recruitment and retention of a diverse and inclusive workforce; and support for an environment of respect for different viewpoints. We measure our progress and hold ourselves accountable.

We renew this commitment not only because it is moral and ethical, but for the fundamental reason it is the right thing to do. Should you have concerns related to equity, diversity or inclusion practices at BCH, you may contact your Department Leadership, Human Resources, or the Anonymous Compliance Hotline (888-801-2805).

Facilities Management

The Facilities Management Manual describes space assignment, use, and management of Emergency Response Floor Plans. The plans describe evacuation routes, and identify room numbers and areas. The Contractor Safety Guide supports project managers and construction personnel in safe and efficient building operations.

Finance

The Finance Department provides accurate, timely, and clear financial and statistical information that supports the organization's understanding of its operations. Additionally, the department efficiently processes financial transactions. The Finance Manual describes financial practices that impact the organization and its employees. The Finance Employee Manual describes departmental-specific organization, operations, and policies. The Health Information Systems Manual describes patient financial transactions such as pre-approval and billing.

Hospital Trust

The Trust secures philanthropic support for Boston Children's patient care, research, education and community service initiatives. If you need support for an important program, the Trust can help identify sources and partner with you to bring in philanthropic funding.

Human Resources

Human Resources helps the organization foster an environment that is respectful and supportive. It accomplishes this by recruiting and retaining bright people who have the opportunity to excel as individuals and as teams. All employees should be aware of the information in the Human Resources Manual, which covers: Sexual Harassment; Discipline, Discharge and Standards of Conduct; and Diversity, Equal Employment Opportunity and Affirmative Action. Additionally, the Clinician Administrative

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Manual provides standards and expectations of professional behavior. Your benefit information can also be accessed at bostonchildrenshr.org.

Information Services

The Information Services Department (ISD) delivers top quality support and develops innovative solutions. We build security systems to prevent the loss, modification or misuse of patient and organizational information. The ISD Manual describes acceptable use of computer and network resources. The ISD Employee Manual describes departmental-specific organization, operations, and policies. It also describes the functions and responsibilities of the various areas in Boston Children's overall computing environment. ISD also oversees the Patient Health Information Manual that specifies patient medical care documentation, as well as access, storage and release of medical records.

Marketing and Communications

The Marketing and Communications Department performs marketing, communications, research, media relations and public affairs functions. It is responsible for branding and promoting Boston Children's and the community that we serve. We develop programs that encourage patients to seek clinical care at all of our locations by raising visibility and strengthening our reputation.

Patient Care Services

Patient Care Services covers various departments that provide excellent patient care. The following manuals guide them to ensure the delivery of quality care:

- **Biomedical Engineering Manual** – addresses medical equipment management, safety, and performance measures.
- **Clinician Administrative Manual** – describes clinician governance, staffing, and personnel policies.
- **Graduate Medical Education Manual** – outlines standards and policies for the program.
- **Infection Prevention and Control Program Manual** – reduces the impact of infectious diseases or organisms on patients, personnel, and visitors. This helps to ensure that Boston Children's is an environment free from hazards, which may result in injury or illness.
- **Emergency Response Manual** – works in conjunction with the Hospital Incident Command System (HICS) to identify responders' roles, responsibilities, and strategies during an emergency. It contains site-specific response plans to incidents that disrupt operations.
- **Laboratory Medicine Manual** – contains policies and procedures for the Department of Lab Medicine and Point of Care Testing.
- **Nursing Orientation Handbook** – describes training for new nursing staff, and the parameters for safe, consistent, and quality interventions.
- **Pathology Manuals** – describe standards for pathology and cytopathology services provided by the Pathology Department.
- **Patient Care Manual** – contains polices related to safe patient care and specialty care content for: Ambulatory Care, Behavioral Health Care, Cardiovascular and Critical Care Manual, Case Management, Child Protection, Clinical Operations, Nutrition, Emergency Services, NICU Care, Pediatric Oncology Care, Peri-Operative Care, Radiation Sedation, Respiratory Care, and

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Trauma Program Care.

- **Patient Experience Manual** – is the sum of all interactions, shaped by our organization's culture, that influence patient perceptions across the continuum of care.
- **Pharmacy and Therapeutics Manual** – includes policies regarding medication prescription, preparation, administration and pharmacy monitoring activities to ensure safe medication use.
- **Radiology Department Manual** – describes department organization, operations, policies, and patient care information.

Program for Patient Safety and Quality

We are committed to providing the highest quality pediatric care to all patients and families. The Program for Patient Safety and Quality (PPSQ) provides leadership oversight and coordination for activities involving Patient Safety & Quality within BCH. The primary goal of our institution-wide program is to ensure that all care is safe, highly effective, patient-centered, timely, efficient and equitable. PPSQ also focuses on Clinical Regulatory Compliance and ensures organizational compliance by embedding regulatory standards into practice. Clinical Regulatory Readiness implements processes, prepares for surveys and inspections, and works with regulators and stakeholders to remedy and address any necessary corrective actions.

Purchasing

Purchasing enforces the Vendor Management Policy that governs relationships with outside entities that are seeking to do business with the organization. It prohibits acceptance of personal gifts and establishes guidelines for relationships with representatives (like those from pharmaceutical and medical device companies).

Research Administration

Research Administration supports our research mission of providing high quality care and leading pediatric institutions in the development and education of research personnel. The Institutional Review Board (IRB) is committed to safeguard the rights and welfare of all children, adolescents, adults and family members who volunteer to participate in research. The Research Administration Employee Manual describes departmental-specific structure, operations, and policies and procedures. The Education and Quality Improvement Program (EQuIP) has a manual that outlines specific standard operating procedures. The Radiation Safety Manual has guidelines that promote the safest conditions possible for staff, patients, contractors, visitors, and the environment. The Sponsored Research Manual has cost policies that Researchers need to follow. The Technology and Innovation Development Office maintains the Intellectual Property Policy that outlines development of research and clinical discoveries that benefit the public, encourage inventorship, and build our research endowment. The Research Misconduct Policy upholds the primary goal of fostering a research environment that promotes a culture of responsibility, integrity, and ethical behavior.

Support Services

Support Services include Materials Management, Operations, Parking & Transportation, Environmental Health & Safety, Safety, and Security. The Chemical Hygiene Plan contains policies and procedures that ensure safe use, storage, and disposal of chemicals and reflects our commitment to providing a safe

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and healthful environment for all patients, visitors, staff and volunteers. The Parking & Commuter Services Manual provides information for leased parking spaces and daily parking vouchers, bicycle cages, and commuter programs (like T-passes and Commute Works). The Environmental Health & Safety Manual outlines the responsibilities of the Safety Officer On-Call as well as other employees that are involved in managing safety incidents. The Security, Operations, & Reception Manual promotes a safe environment through protection of individuals and property at Boston Children's.

Thank You

Dear Fellow Member of the Boston Children's Community,

We really appreciate that you chose to work at Boston Children's Hospital.

We sincerely hope that you enjoy the time that you spend working here and thank you for your service.

Welcome!

Timothy C. Hogan, JD, FHFMA, CHC
Senior Vice President and Chief Compliance Officer
timothy.hogan@childrens.harvard.edu
857-218-4681

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Approval Signatures

Step Description	Approver	Date

Applicability

Boston Children's Hospital- Policies & Procedures

Chapter / Tags

Anonymous Compliance Hotline, Code of Conduct, Communicating Clearly, High Reliability, Honesty, Integrity, Maintain High Standards, Mission, Paying Attention to Detail, Speaking up for Safety, Values

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EXHIBIT E

Decision to Terminate

Employee: Alan Turner
ID #: 251056
Position: Behavioral Health Counselor
Department: Psychiatry
Manager: Sharon Arnow
Date of Hire: 7/17/2023
Reason: Violation of Policy (specify which policy)
Date: 12/4/23

Alan □urner □as been em□loyed by Boston Children's Hospital since □/1□/23□ In □is □osition of □e□avioral □ealt□ Counselor, Alan is res□onsible for milieu mana□ement and □atient safety in con□unction wit□ □is dele□ated res□onsibilities from RNs□

Current Situation: On 11/9/23 t□ere was a re□ort of ina□□ro□riate be□avior to leaders□i□ by Alan towards a female collea□ue□A review of t□e situation wit□ t□is individual revealed two ot□er concerns of female collea□ues □avin□ ex□erienced similar conduct by Alan□□□e be□avior was initially small tal□, and over time became more □ersonal in nature to w□ic□ eac□ individual describes minor flirtatious actions, touc□in□ of a s□oulder or an unsolicited □u□□ One situation wit□ a female collea□ue □ad become more □ervasive wit□ Alan re□eatedly su□□estin□ to □er t□at t□ey "□et to□et□er" and no need for datin□ but to □ust be to□et□er□

In review of t□is situation wit□ Alan t□is a□□eared to be a □attern of be□avior t□at □e □as demonstrated before, and □e admitted bein□ fired from anot□er institution for t□is be□avior□Alan stated □e was □ust bein□ □imself and offerin□ advice to t□ese individuals□

□□ese be□aviors are a direct violation of t□e □os□ital □iscrimination □ Sexual □arassment □olicy□

Recommended Action□□ue to t□e □ervasive nature of t□e be□avior mana□ement □as recommended termination of em□loyment immediately□

Submitted by□ □□□S□aron Arnow Nurse □ana□er□□□□□□□□□□□□□□□□12/□/23□□□□□□□□□
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EXHIBIT F

Guidelines on 151B:

Sexual Harassment in the Workplace

Introduction

1 Massachusetts Law prohibits sex discrimination in the workplace.2 Sexual harassment is a form of sex discrimination. Sexual harassment is also prohibited in places of public accommodation,3 educational facilities4 and housing.5 These guidelines address sexual harassment in the workplace only.

The standards governing the prohibition of sex discrimination and sexual harassment in the workplace are set forth in Massachusetts General Laws chapter 151B ("chapter 151B"). The Massachusetts Commission Against Discrimination ("MCAD" or the "Commission") issues these guidelines to assist employers, employees, attorneys and the general public in understanding what constitutes sexual harassment, as well as to explain what employees and employers should do to prevent, stop and appropriately respond to sexual harassment. In addition, these guidelines discuss the circumstances under which employers6 and individuals may be held liable for sexual harassment in the workplace.

Sexual Harassment

There are two types of sexual harassment: "quid pro quo" harassment and "hostile work environment" harassment. They may occur independently or concurrently.

Quid Pro Quo Harassment

Chapter 151B defines "quid pro quo" sexual harassment as:

sexual advances, requests for sexual favors, and other verbal or physical conduct of a sexual nature when . . . submission to or rejection of such advances, requests or conduct is made either explicitly or implicitly a term or condition of employment or as a basis for employment decisions 7
To prove a claim for quid pro quo harassment, the employee must establish the following elements:

- That the alleged harasser made sexual advances or sexual requests, or otherwise engaged in conduct of a sexual nature;
- the sexual conduct was unwelcome;
- he or she rejected such advances, requests or conduct; and
- the terms or conditions of his or her employment were then adversely affected.8

or

- That the alleged harasser made sexual advances or sexual requests, or otherwise engaged in conduct of a sexual nature;
- the sexual conduct was unwelcome;

- he or she submitted to such advances, requests or conduct; and when he or she submitted to the unwelcome sexual conduct, he or she did so in reasonable fear of adverse employment action.9

Quid pro quo harassment occurs when an employee with authority or control over the terms and conditions of another employee's work offers her10 a work benefit or advantage in exchange for sexual favors or gratification.11

Conversely, if an employee is denied a work benefit or advantage due to her refusal to respond to, or rejection of, requests for sexual favors or gratification, she is subjected to quid pro quo harassment.

Thus, either submission to, or rejection of, unwelcome sexual advances may result in quid pro quo harassment if the terms or conditions of one's employment are impacted.12 Examples of such impact may include but are not limited to: termination; demotion; denial of promotion; transfer; alteration of duties, hours or compensation; or unjustified performance reviews. 13

Once the complainant establishes a prima facie case, the burden of production, as opposed to the burden of proof, shifts to the respondent to articulate a legitimate, non-discriminatory reason for the adverse employment action taken, supported by credible evidence.14

If the respondent meets its burden of production, the complainant must prove that the reasons offered by the respondent were not its true reasons, but were a pretext for discrimination.15

For example, a complainant may meet her initial burden by showing that she was fired soon after she turned down her supervisor's request for a date. The request for a date would constitute the unwelcome advance. If there is sufficient closeness in time between the complainant's rejection of her supervisor's request and her firing, this may create an inference of causation.16 The employer would then have to articulate a legitimate non-discriminatory reason for its adverse action supported by credible evidence.

For example, if the respondent credibly asserts that the complainant was terminated for poor job performance, the burden remains with the complainant to prove, by a preponderance of the evidence, that the respondent's conduct was motivated by her rejection of her supervisor's advance.17 This may be done by proving that the respondent's articulated reason is false. 18

Hostile Work Environment 19

Chapter 151B defines "hostile work environment" harassment as:

sexual advances, requests for sexual favors, and other verbal or physical conduct of a sexual nature when . . . such advances, requests or conduct have the purpose or effect of unreasonably interfering with an individual's work performance by creating an intimidating, hostile, humiliating or sexually offensive work environment. 20
In a hostile work environment case, the complainant must prove:

- she was subjected to conduct of a sexual nature;
- the conduct of a sexual nature was unwelcome;

- the conduct of a sexual nature had the purpose or effect of creating an intimidating, hostile, humiliating or sexually offensive work environment; and
- the conduct unreasonably interfered with complainant's work performance or altered the terms and conditions of the complainant's employment.21

Conduct of a Sexual Nature

Examples of conduct that might create a hostile work environment include: inappropriate touching; sexual epithets, jokes, gossip, sexual conduct or comments; requests for sex; displaying sexually suggestive pictures and objects; and leering, whistling, or sexual gestures. 22 Harassing conduct need not be motivated by sexual desire in order to constitute sexual harassment.23

Welcomeness

The law does not proscribe all conduct of a sexual nature. Only unsolicited and unwelcome conduct may create a hostile work environment. When the employee initiates conduct of a sexual nature or is a willing participant in a sexually charged environment, she may not be the victim of sexual harassment. Whether the conduct was "welcome" does not turn on whether the complainant's behavior was "voluntary." When an employee only submits to harassing behavior to avoid being targeted further, to cope in a hostile environment, or because participation is made an implicit condition of employment, she is not considered to have welcomed the conduct.24 The employee's rejection of, or failure to respond positively to, suggestive comments or gestures may demonstrate unwelcomeness. The fact that the employee may have infrequently joked with the alleged harasser does not demonstrate that the alleged harasser's entire course of conduct was welcome.25An employee need not communicate her objection to harassing conduct to demonstrate its unwelcomeness. 26

Conduct Creating a Hostile Work Environment

In order to rise to the level of creating a hostile work environment, the conduct must be hostile, intimidating, humiliating or offensive both from an objective and a subjective perspective.27 An employee who does not subjectively perceive the behavior at issue as hostile, intimidating, humiliating or offensive is not a victim of sexual harassment within the meaning of the law, even if other reasonable individuals would consider such behavior to be so.28On the other hand, an employee who subjectively finds behavior to be hostile, intimidating, humiliating or offensive when it is not objectively so, is not a victim of a hostile work environment under chapter 151B. Thus, for example, if a female employee is faced with requests for sexual favors, the question becomes whether a reasonable person in her position would find the conduct offensive and whether she actually found the conduct offensive.29

In determining whether conduct is objectively offensive, the Commission looks to whether the conduct is severe or pervasive. In order for conduct to be considered pervasive, a complainant must prove that she was subjected to "a steady barrage of opprobrious [sexual] comment or abusive treatment."30 Such treatment can involve a combination of physical and verbal conduct, e.g., unwanted groping or touching combined with profanity or sexual innuendo. Sexual harassment experienced by others in the workplace may also be relevant to the assessment of the conduct's pervasiveness.31

In some circumstances, a hostile environment may be established based on a single incident, due to its severity, despite the fact that the conduct is not frequent or repetitive.32Moreover, purely verbal

conduct, without a physical component, may be severe or pervasive enough to create a hostile work environment. However, minor, isolated conduct does not constitute sexual harassment.33 "A few isolated remarks over a period of time" are generally insufficient to meet the pervasiveness standard.34 Chapter 151B is not a clean language statute and does not prohibit all use of profane or offensive language.35

Conduct that Interferes with an Individual's Ability to Perform Her Job

Proof of a hostile work environment claim requires a showing that the unwelcome sexual conduct created an impediment to an employee's full participation in the workplace, altered the terms and conditions of her employment, or unreasonably interfered with her work performance.36While not all offensive or inappropriate conduct will create such an impediment,37one's working conditions may be altered without a showing of a tangible job detriment.38Thus, an employee may seek recovery for hostile environment sexual harassment even if she has not suffered an adverse job action such as termination, suspension, or demotion.39

Whether conduct interferes with an individual's ability to perform her job is essentially a question of fact based on the totality of the circumstances, which include the nature, severity and pervasiveness of the conduct and the psychological harm to the employee. Conduct that interferes with an employee's ability to do her job need not necessarily cause severe psychological harm or emotional distress to be actionable.40

Same-Sex Sexual Harassment

Sexual harassment can occur between individuals of the same gender. The same standards that apply to sexual harassment between individuals of the opposite sex apply to harassment cases involving individuals of the same gender. 41Under a same-sex sexual harassment claim, the sexual orientations of the parties are irrelevant, as the harassing conduct need not be motivated by sexual desire to be actionable.42 In addition, there is no requirement under chapter 151B that a complainant prove the conduct was motivated by his or her gender. 43

Sexual Harassment Outside of the Workplace

Chapter 151B may apply to harassment that occurs between co-workers that takes place outside the workplace. When the conduct complained of occurs outside of the workplace, the Commission may consider the following factors in assessing whether the conduct constitutes sexual harassment:

- whether the event at which the conduct occurred is linked to the workplace in any way, such as at an employer-sponsored function;44
- whether the conduct occurred during work hours;45
- the severity of the alleged outside-of-work conduct;
- the work relationship of the complainant and alleged harasser, which includes whether the alleged harasser is a supervisor and whether the alleged harasser and complainant come into contact with one another on the job; 46
- whether the conduct adversely affected the terms and conditions of the complainant's employment or

- impacted the complainant's work environment.47

Employer Liability

Sections 4(1) and 4(16A) of chapter 151B provide the statutory basis for employer liability in cases of sexual harassment.

Section 4(1)

Section 4(1) states in relevant part:

It shall be an unlawful practice:

For an employer, by himself or his agent, because of the ... sex of any individual to ... discriminate against such individual in compensation or in terms, conditions or privileges of employment.

Section 4(16A)

Section 4(16A) states:

It shall be an unlawful practice:

16A. For an employer, personally or through its agents, to sexually harass any employee.

Employers are Liable for Harassment by Persons with Supervisory Authority

An employer is liable for the sexual harassment of employees by managers and persons with supervisory authority, regardless of whether the employer knows of the conduct.48 Because Massachusetts courts have determined that the Legislature intended that an employer be liable for discrimination committed by those on whom it confers authority, the courts have adopted the theory of vicarious liability in harassment cases. 49An employer is liable for the actions of its managers and supervisors because they are conferred with substantial authority over subordinates and are thus considered agents of the employer. 50 In some circumstances, an employer may be liable for the actions of a supervisor, even if that supervisor does not have direct supervisory authority over the Complainant. 51

Factors the Commission will consider as indications of supervisory authority include, but are not limited to

- Undertaking or recommending tangible employment decisions affecting an employee;
- Directing activities, assigning work and controlling work flow;
- Hiring, firing, promoting, demoting or disciplining;
- Altering or affecting an employee's compensation or benefits;
- Evaluating an employee's work load;
- Distributing necessary supplies and tools;

- Giving directions and verifying and fixing mistakes;
- Assisting employees in assigning tasks; and
- Monitoring and evaluating work performance.52

The employer may be vicariously liable for sexual harassment even if the alleged harasser is not formally designated as a supervisor and even if a supervisor lacks actual authority, under the doctrine of apparent authority. 53 Liability under these circumstances exists when the harasser holds himself out to the employee as having supervisory authority over the employee.54 The employee's belief that the harasser has authority over her, to the extent that it is reasonable, may be a significant factor in determining the existence of apparent authority.55

Employer Is Liable for Sexual Harassment in Other Circumstances

An employer may also be liable for sexual harassment committed by persons without actual or apparent supervisory authority, such as co workers.56 The complainant must show that the employer either knew57 or should have known 58about the harassing conduct and failed to take prompt, effective and reasonable remedial action. 59 The respondent's obligation to take remedial action is discussed in greater detail in §VI(F), infra.

An employer may also be liable for the sexual harassment of its employees by certain non-employees, such as customers, 60 patients,61 clients, 62 independent contractors or other acquaintances. 63 An employer may be held liable for the unlawful conduct of such non-employees when the employer knew or should have known about the conduct and failed to take prompt, effective and reasonable remedial action. The primary difference between employer liability for harassment perpetrated by co-workers and harassment committed by non-employees lies in the ability of the employer to control the conduct of the non-employees. The greater the employer's ability to control the non-employee's conduct, the more likely it will be found liable for that person's unlawful harassment. 64

Individual Liability for Sexual Harassment

An individual may be held liable for sexual harassment as an employer under M.G.L. c. 151B, § 4(1) and § 4(16A), or under M.G.L. c.151B, §§4(4A) and 4(5), which specifically prohibit "any person" from engaging in certain types of discriminatory conduct.65

Individuals May Be Liable as the Employer

When an individual is the employer, rather than merely an agent of the employer, the individual may be liable under chapter 151B, §4(1) and §4(16A), which prohibit unlawful sexual harassment on the part of an employer. Depending on the size, nature and form of the business, an individual may be so closely identified with the business entity that the individual is personally liable as the employer.66 This may apply to principals, owners, presidents or partners in a business.67

Individuals May Be Liable Under Chapter 151B, § 4(4A)

Chapter 151B, §4(4A) states that it is an unlawful practice:

for any person to coerce, intimidate, threaten or interfere with another person in the exercise or enjoyment of any right granted or protected by this chapter.(emphasis supplied).
Individuals may be subject to liability under §4(4A) for engaging in sexually harassing conduct.68 Both supervisors and co-workers may be liable under this section for engaging in sexually harassing conduct. 69 Furthermore, the Commission has held that an individual may be liable even in circumstances where the employer is not subject to liability. 70 Section 4(4A) even reaches the conduct of a third party, non-employee who sexually harasses an employee.71

Individuals May Be Liable Under Chapter 151B, § 4(5) ("Aiding and Abetting" Liability)

Chapter 151B, §4(5) states that it is an unlawful practice:

for any person, whether an employer or an employee or not, to aid, abet, incite, compel or coerce the doing of any of the acts forbidden under this chapter or to attempt to do so.
Under the language of §4(5), any individual - including employees of respondents and third parties - who actively perpetrates or assists another in acts prohibited by chapter 151B can be held separately liable as an aider and abettor.72The tripartite standard for determining "aider and abettor" liability is:

- The wrongful act must be separate and distinct from the underlying claim or an act in furtherance of the underlying claim;
- The aider and abettor shared an intent to discriminate not unlike that of the alleged principal offender; and
- The aider and abettor knew of his or her supporting role in an enterprise that deprived an individual of a right guaranteed under M.G.L. c. 151B.73

Inaction by an employee may, under certain circumstances, give rise to individual liability under §4(5). For liability to attach in this circumstance, the individual must:

- have knowledge of ongoing sexual harassment;
- have an obligation and the authority to investigate and/or take remedial action;
- intentionally fail to take such action; and
- contribute to the complainant's injury by failing to act.74

"[I]n situations where the inaction of the employee results from 'deliberate indifference,' and not mere inattention or negligence, such nonfeasance 'is not mere inaction, but a designed and willful act of forbearance in a situation where action is required.'" 75 However, if the employee has no duty to intervene to stop the harassment and is not in a position to do so, he will not be subject to liability under §4(5). 76

An individual may only be liable as an aider or abettor when there is evidence of a joint enterprise between more than one participant.77 The individual must have specific knowledge of his or her supporting role in the unlawful enterprise. 78 An individual may engage in a joint enterprise with the

"fictional" legal entity of the respondent corporation - which possesses all of the legal attributes of a natural person - if that individual is in a position to subject the employer to vicarious liability. 79

Employers' Sexual Harassment Policies and Complaint Procedures

In Massachusetts, the law requires employers with six or more employees to adopt a written policy against sexual harassment.80 The employer's policy must include notice to employees that sexual harassment in the workplace is unlawful and that it is unlawful to retaliate against an employee for filing a complaint of sexual harassment.81The policy should also assert the employer's commitment to investigate any complaint of sexual harassment. The Commission has prepared a Model Sexual Harassment Policy and a poster.82

Sexual Harassment Policy

The Commission recommends that an employer's policy include, at a minimum, all the requirements enumerated in chapter 151B and all the components of the Model Policy, as follows:

- a statement that sexual harassment in the workplace is unlawful;
- a statement that it is unlawful to retaliate against an employee for filing a complaint of sexual harassment, or for cooperating in an investigation of a complaint of sexual harassment;
- a description and examples of sexual harassment
- a statement of the potential consequences for employees who are found to have committed sexual harassment;
- a description of the process for filing internal complaints about sexual harassment and the work addresses and telephone numbers of the person or persons to whom complaints should be made; and
- the identity of the appropriate state and federal employment discrimination enforcement agencies and instructions as to how to contact such agencies.

Employers should specifically prohibit the dissemination of sexually explicit voice mail, e-mail, graphics, downloaded material or websites in the workplace and include these prohibitions in their workplace policies. 83 An employer must present new employees with a copy of the employer's policy upon commencement of employment and provide all employees with an individual written copy of the policy on a yearly basis.84 Employers should also post the policy in a conspicuous area in the workplace.

Sexual Harassment Training and Education

While not a requirement, chapter 151B encourages employers to conduct education and training programs on sexual harassment for all employees on a regular basis.85 Employers are further advised to conduct additional training 86 for supervisory and managerial employees, which should address their specific responsibilities as well as the steps that such employees should take to ensure immediate and appropriate corrective action in addressing harassment complaints. 87This is significant because employers are vicariously liable for the conduct of those persons that they place in supervisory positions.88

Employers should also train employees how to recognize and report incidents of sexual harassment. In claims alleging sexual harassment, an employer's commitment to providing anti-harassment training to its workforce may be a factor in determining liability or the appropriate remedy.89

Sexual Harassment Complaint Procedure

The following suggestions regarding how to draft an appropriate complaint procedure and conduct an investigation of a sexual harassment claim are advisory in nature, rather than mandatory.

Employers Should Designate Person(s) to Receive Complaints of Sexual Harassment

In its sexual harassment policy, an employer should designate one or more individuals as the person(s) to whom employees should report any complaints of sexual harassment. The person(s) selected should be knowledgeable and sensitive to the issues.90These typically include managers, supervisors, human resource personnel, principals of the employer or in-house counsel. The full names of these individuals, together with their work addresses and telephone numbers, should be included in the policy provided to employees.

The employer's internal complaint procedure should also be calculated to encourage complainants to come forward by:

- designating more than one individual to receive complaints;
- designating individuals of both sexes to receive complaints;
- designating individuals of both sexes to receive complaints;91
- allowing complaints to be communicated orally; and
- allowing a complainant to report allegations to someone other than his or her supervisor.

It is also advisable for an employer to identify the person(s) and/or the department within the organization that will be authorized to initiate an internal investigation into a complaint of harassment. Any employee who receives a complaint of sexual harassment, or is made aware of any sexually harassing behavior, should immediately report it to the designated person(s) and/or the designated department.

Under no circumstances should an employer:

- require an employee to complain directly to the person alleged to have engaged in the sexual harassment;
- insist that filing a complaint within the company is a prerequisite to filing or pursuing a complaint of sexual harassment with the MCAD or the United States Equal Employment Opportunity Commission ("EEOC");
- instruct an employee not to file a complaint with the MCAD or EEOC; or
- instruct an employee not to cooperate with the MCAD or EEOC in an investigation.

Duties of Persons Receiving Complaints of Sexual Harassment

The individuals designated to receive complaints of sexual harassment on behalf of the employer should document the receipt of any such complaints. It is advisable for that person to maintain the records in a way that allows for the identification of repeat offenders.

Employers should instruct recipients of sexual harassment complaints to inform complainants and alleged perpetrators that they will:

- keep the complaint confidential to the extent practicable under the circumstances;
- conduct a prompt, neutral investigation into the allegations; and
- not tolerate any form of retaliation against the complainant for having complained of sexual harassment.

Investigating Sexual Harassment Complaints

Preliminary Issues

In general, the employer should always investigate a complaint of sexual harassment as soon as practicable, even if an employee asks that it not investigate his or her claim. Employers should investigate any claim involving physical violence immediately. The nature and duration of the investigation will depend on the circumstances of the complaint, including the type, severity and frequency of the alleged harassment. 92

Conducting the Investigation

Employers should investigate allegations of sexual harassment in a fair and expeditious manner, in a way that maintains confidentiality to the extent practicable. Employers should inform complainants that they have a legal duty to investigate allegations of sexual harassment, and that, while the matter will be kept as confidential as possible, it may not be possible to withhold the complainant's identity from the alleged harasser. The employer's investigation should generally include interviews of the complainant, the alleged harasser, witnesses, individuals whom any of the foregoing identify as having knowledge of potential relevance to the allegations, and anyone else whom the employer believes may have such knowledge.93 These interviews should be conducted in a way that protects the privacy of the individuals involved to the extent practicable under the circumstances. They should also be conducted in person where possible.

The investigator should inform each interviewee, as well as any other individual apprised of the investigation, that the investigation is confidential and should not be discussed with co-workers. The investigator should further inform them that the employer will not tolerate any retaliation against the complainant or anyone else who cooperates with the investigation. The investigator should also prohibit interference with or obstruction of any investigation by the MCAD or EEOC into the allegations.

The employer's investigation should also include a review of any documents, journals, recordings, photographs, voice mails, e-mails, telephone records, or other items that may be relevant to the allegations of harassment. 94

The Investigative File

The investigator should take notes during interviews, or soon thereafter, for the purpose of maintaining accurate records. The investigator should create and maintain a confidential investigative file separate from personnel files. The file should include any materials relevant to the investigation, including but not limited to interview notes, relevant employment documents, journals, recordings, photographs, voice mails, e-mails, telephone records, or other items pertaining to the allegations or the investigation into them.

Special Issues Regarding the Investigation

Confidentiality
The employer should ensure that the investigation is kept as confidential as possible, by communicating information about the investigation only to those that need to know about it. An employer should not promise absolute confidentiality to the complainant, the alleged harasser or other witnesses, because such a promise may obstruct the employer's ability to conduct a fair and thorough investigation. Generally, the complainant and the alleged harasser should be kept informed of the status of the investigation during the process.

Interim Measures Pending the Outcome of the Investigation
During the investigation, it may be necessary or advisable for the employer to take measures to separate the alleged harasser from the complainant. These measures should be carefully crafted to minimize the chance that the alleged harasser will either continue to harass the complainant or will retaliate against her. The employer must also ensure that the measures themselves do not amount to retaliation against the complainant. The employer should consider a number of factors in deciding what interim measures to take, including, but not limited to, the following:

- the expressed wishes of the complainant;
- the nature and extent of the allegations;
- the personal safety of the complainant;
- the number of complainants;
- whether the alleged harassment is of an ongoing nature;
- the behavior of the alleged harasser; and
- whether the alleged harasser has an alleged or actual history of engaging in harassment.

Consideration of these factors may lead the employer to decide that certain interim measures are necessary and/or advisable. Such measures might include, but are not limited to:

- placing the alleged harasser on administrative leave;
- placing the complainant on administrative leave, if the complainant so requests;
- transferring the alleged harasser, or the complainant if she requests, to a different

- area/department or shift so that there is no further business/social contact between the complainant and the alleged harasser;
- instructing the alleged harasser to stop the conduct; and
- eliminating the alleged harasser's supervisory authority over the complainant.

During the investigation, the employer has a duty to take the necessary steps to eliminate from the workplace the harassment about which the complainant has complained.95 The fact that it may be burdensome for the employer to take such action does not diminish this duty.96 The employer should monitor any interim measures that it takes throughout the investigation. Monitoring may include assessing whether the interim measures meet the goals of preventing ongoing harassment, protecting the safety of the parties and preventing retaliatory conduct.

Reaching a Determination

After the employer's investigation is complete, the investigator should prepare a final written report documenting his or her findings. Generally, the investigator's report should detail the steps the investigator took in examining the complainant's allegations and should explain any conclusions the investigator has made. The employer should inform the complainant and the alleged harasser of its findings in the matter. If the employer concludes that sexual harassment has occurred,97the employer must take prompt and appropriate remedial action designed to end the offending conduct and prevent future harassing conduct.98 Regardless of the investigator's findings, the employer should make follow-up inquiries to ensure that the conduct has not resumed and that neither the complainant nor any witnesses interviewed during the investigation has suffered any retaliation.

Appropriate Remedial Action

When an employer concludes that sexual harassment has occurred, the employer must take prompt remedial action designed to end the harassment and prevent future harassment. What constitutes appropriate remedial action depends upon the circumstances. Appropriate remedial action should reflect the nature and severity of the harassment, the existence of any prior incidents, and the effectiveness or lack thereof of any prior remedial steps.

Generally, remedial action consists of the following:

- promptly halting any ongoing harassment;
- taking prompt, appropriate disciplinary action against the harasser;
- taking effective actions to prevent the recurrence of harassment, including conducting a sexual
- harassment training where appropriate;99 and
- making the complainant whole by restoring any lost employment benefits or opportunities.100

Whether the employer has taken prompt and appropriate remedial action in a given case depends upon many factors, including the timeliness of the actions and whether, given the circumstances, the actions were reasonably likely to stop the conduct and prevent it from recurring. The inquiry into whether the

employer took appropriate action is focused primarily on whether the remedial action ultimately succeeded, taking into consideration whether, under the circumstances, the action was reasonably calculated to succeed.101 The efficacy of the action is not measured by whether the complainant feels that justice has been achieved, but whether the behavior that gave rise to the complaint has ceased and does not threaten to recur.102

Statute of Limitations

Timeframe for Filing a Claim

Currently, chapter 151B, § 5 requires that a charge of discrimination be filed with the Commission within six months of the alleged discriminatory act. However, the statute was amended on August 7, 2002 and, beginning November 5, 2002, the time period will be amended to 300 days. 103

The filing requirements are to be interpreted broadly to give effect to the law's broad remedial purposes.104 Pursuant to the Commission's Regulations at 804 CMR §1.10(2), the limitations period will not bar the filing in instances where the facts of a charge allege that the unlawful conduct was of a continuing nature (as discussed below) or "when pursuant to an employment contract, an aggrieved person enters into a grievance proceeding concerning the alleged discriminatory act(s) within six months (or 300 days depending on the applicable period) of the conduct complained of and subsequently files a complaint within six months (or 300 days) of the outcome of such proceeding(s)." The regulation further provides that the statutory requirement is not a bar where the aggrieved person enters into an agreement to mediate a dispute under M.G.L. c. 151B and files the complaint within twenty-one days after the conclusion of mediation.

In addition, the filing deadline is also subject to equitable tolling. Equitable tolling has been found to apply only in the following narrow circumstances: (1) the complainant is excusably ignorant about the statute of limitations; or (2) the respondent/employer or the Commission affirmatively misleads the complainant.105 Generally, the time period under chapter 151B is triggered once the complainant knows or should know that he or she is being discriminated against.106

The Continuing Violations Doctrine

The applicable filing deadline will not prevent employees from pursuing claims when the conduct complained of is of a continuing nature. 107This situation may occur if the case involves a pattern of conduct, the cumulative effect of which results in a hostile work environment over time, as opposed to a distinct job action that takes place on a specific date. This exception recognizes that some claims of discrimination involve a series of related events that have to be viewed in their totality in order to assess adequately their discriminatory nature and impact.108 This is because "incidents of sexual harassment serious enough to create a work environment permeated by abuse typically accumulate over time," and any one incident, viewed in isolation, may not be serious enough to constitute harassment.109However, viewed cumulatively, "the seemingly disparate incidents may show a prolonged and compelling pattern of mistreatment that have forced a plaintiff to work under intolerable, sexually offensive, conditions." 110

A continuing violation may be of a serial or a systemic nature.

Serial Violation

A "serial" continuing violation exists when: (a) at least one instance of sexually harassing conduct occurs within the limitations period; (b) the timely and untimely acts are recurrent, related to each other, and stem from a common discriminatory animus; (c) the complainant's delay in filing the charge as to the untimely events was not unreasonable.

If a continuing violation exists, the applicable filing period does not begin to run until the occurrence of the last act of discrimination. 111 Under these circumstances, harassing events occurring outside the statute of limitations may be considered timely. Therefore, if the complainant is able to establish the existence of a continuing violation, she may be able to recover damages for otherwise untimely acts in addition to damages for the timely conduct.112 The MCAD will find the complainant's sexual harassment claim timely if the following conditions are satisfied:

- At Least One Instance of Sexually Harassing Conduct Within the Applicable Limitations Period
  An employee must establish that at least one instance of sexually harassing conduct occurred within the applicable limitations period. The conduct within the limitations period need not, standing alone, have created a hostile work environment. 113 However, the fact that an employee may have ongoing distress within the applicable time period as a result of conduct occurring prior to that period will not suffice.114

- The Conduct Must Be Substantially Related and Recurrent
  In order for an employee to meet this prong, it must be shown that the timely act "substantially relates to earlier incidents of abuse, and substantially contributes to the continuation of a hostile work environment, such that the incident anchors all related incidents, thereby making the entirety of the claim for discriminatory conduct timely."115 In other words, both the timely and untimely conduct must stem from a common discriminatory animus. 116 Factors the Commission considers are the similarity of the acts, whether certain conduct is repeated, the nature of the timely and untimely conduct, the amount of time between incidents and the time period over which the conduct is alleged to have occurred. If a substantial relationship is found, the Commission may find that the complainant's untimely allegations of sexual harassment are actionable. 117 If the timely and untimely conduct are not linked, the conduct outside the limitations period will not be actionable. For example, hateful looks, without more, may not be found to be sufficiently related to prior, untimely acts of sexual harassment. 118

- The Complainant's Delay in Filing the Charge Must Not Be Unreasonable
  The complainant may seek damages for conduct occurring outside the limitations period, unless she "knew or reasonably should have known that her work situation was pervasively hostile and unlikely to improve, and, thus, a reasonable person in her position would have filed a complaint with the MCAD before the statute ran on that conduct."119 If the delay as to the earlier events is objectively unreasonable, she may still file a claim as to the timely events and use the untimely events as background evidence. 120

Systemic Violations

A systemic violation occurs when an ongoing discriminatory policy or practice of the employer exists. In order to be rendered timely, the complainant need not establish that the discriminatory act has occurred within the statute of limitations period, rather that the discriminatory policy that affected the complainant continued into the limitations period.121

Constructive Discharge

Constructive discharge occurs when a complainant resigns or leaves a job due to working conditions so intolerable that the law treats the resignation as a discharge. Constructive discharge is not a required element of a sexual harassment claim, but offers an additional basis for damages in connection with such a claim. An employee alleging sexual harassment may prove constructive discharge by showing that she left her job under circumstances where a reasonable person in her position would have felt compelled to resign.122

Sexual harassment that results in constructive discharge may involve both verbal and physical conduct - e.g., unwanted sexual comments combined with unwelcome touching.123 However, verbal conduct alone, such as grossly offensive language, can also cause a constructive discharge. An employee who has been subjected to sexual harassment may establish a claim of constructive discharge by demonstrating that it was unlikely that the discriminatory environment would be properly remedied within a reasonable time period. The likelihood of establishing a claim of constructive discharge increases the longer the harassment persists, particularly where the employee has complained of the harassment and no or inadequate remedial action has been taken.

Constructive discharge can occur even if the harasser does not act with the specific intent of forcing the complainant to resign from her job.124 A claim of constructive discharge under chapter 151B does not arise, however, when the complainant resigns due to general dissatisfaction with the workplace or as a result of other conduct that does not violate chapter 151B.125

An employee who is subjected to sexual harassment must pursue reasonable alternatives to quitting, such as filing an internal complaint, in order to establish constructive discharge. 126 Determining whether there are reasonable alternatives to quitting is a fact-specific inquiry. For example, an employee who is sexually harassed by the president of the company may not be required to complain to a Human Resources representative subordinate to the president in order to establish constructive discharge. Moreover, if there is no human resources department or policy regarding how to address a complaint of discrimination, it may not be reasonable to expect an employee who is being harassed by his/her supervisor to file a complaint. It may, however, be reasonable to expect an employee in this situation to make clear to the offending party that the sexually harassing behavior is unwelcome and request that it stop.

As a general matter, where avenues for filing an internal complaint exist, if an employee resigns before the employer has had a reasonable opportunity to investigate and address the allegation of harassment, the resignation is less likely to be determined to be a constructive discharge. Where the complainant makes an internal complaint and the employer fails to respond adequately, constructive discharge may occur. 127 By contrast, responding to allegations of harassment in a prompt, effective, non-retaliatory manner may prevent a finding of constructive discharge.128

Retaliation

Neither an employer nor any person may retaliate against an individual who alleges sexual harassment. Chapter 151B, §4(4) prohibits any person or employer from taking adverse action against a person "because he [or she] has opposed any practices forbidden under [chapter 151B] or because he [or she] has filed a complaint, testified or assisted in any proceeding under [chapter 151B]."129 In order to prove retaliation, a complainant must show that: (A) she engaged in protected activity; (B) her employer knew

of this protected activity and acted adversely against her; and (C) a causal nexus exists between the adverse action and the protected activity.130

Protected Activity

Protected activity may include, but is not limited to, such actions as:

- speaking to someone at the MCAD, EEOC or other civil rights or law enforcement agency, or to
- an attorney about the possibility of filing a claim of discrimination against the employer;
- filing a complaint at the MCAD or EEOC against the employer;
- filing a complaint in court;
- talking to an MCAD or EEOC investigator about another employee's charge of discrimination against the employer;
- testifying as a witness concerning a claim of harassment against the employer;
- complaining to management or filing an internal complaint of harassment;
- asking a supervisor or co-worker to stop engaging in harassing conduct;
- cooperating with an internal investigation of a sexual harassment complaint; or
- meeting with co-workers to discuss how to stop sexual harassment in the workplace.

In order to prove protected activity, a complainant must demonstrate that she "reasonably and in good faith believed that the [employer] was engaged in wrongful discrimination and that [s]he acted reasonably in response to [her] belief." 131 A complainant need not prevail on her sexual harassment claim to prove a retaliation claim. 132

In addition, the way in which a complainant expresses her opposition to the harassing conduct must also be reasonable. For instance, physical violence or threats of physical violence may be considered too extreme a response to be considered reasonable.133 By contrast, conduct such as reporting an incident to a sexual harassment officer, filing a claim at the Commission, providing information in an investigation, or testifying at a proceeding is never considered unreasonable. The Commission's determination as to the reasonableness of a complainant's oppositional conduct will take into consideration the egregiousness of the alleged harassment.

Adverse Action

An employer takes adverse action under §4(4) when it materially disadvantages the complainant with regard to any of the terms or conditions of her employment.134 The term "adverse action" can encompass actions such as:

termination;
denial of promotion;135

demotion in title or duties;
transfer to a less favorable position or location;136 involuntary placement on leave;137
hostile or abusive workplace treatment;138 or
decreasing compensation or benefits.
In addition to actions that are materially disadvantageous, retaliation claims can be based upon allegations of coercion, threats, intimidation, and interference under chapter 151B, § 4(4A), as discussed below.

A complainant must show that her employer knew of her protected activity when it took adverse action. The MCAD has applied a "knew or should have known" standard to impute knowledge of a complainant's protected activity to her employer.139 Certain protected activity such as filing a complaint with the MCAD puts an employer on notice by its very nature. However, such notice would only be imputed to the employer in the presence of proof that the employer had received notice of the MCAD filing.

Causation

A complainant proves causation by showing that her participation in protected activity was "a determinative factor" in her employer's decision to act adversely against her. 140A highly relevant factor in the causation analysis is the proximity in time between the adverse action and the protected activity. 141 The mere fact, however, that adverse action occurred after protected activity does not necessarily show causation.142

Coercion, Intimidation, Threats or Interference

A complainant may also bring a retaliation claim under §4(4A) of chapter 151B if she is subjected to threats, intimidation, or coercion, or her employment is otherwise interfered with because she complained of harassment or assisted or encouraged another who complained of harassment. 143Unlike a §4(4) claim, a §4(4A) claim does not require proof of an adverse employment action. Furthermore, both employees and non-employees can be held liable under this section. For example, an interviewer's threat not to hire an employee may violate §4(4A), even if the interviewer does not have authority to act on the threat.144

Frivolous Claims

The employer has the right to take appropriate disciplinary action against an employee who makes a false or bad faith claim of sexual harassment. In addition, to the extent that any willfully false claim constitutes resistance to or interference with the work of the Commission, the person filing such a complaint may be subject to civil and/or criminal penalties. 145

End Notes

1 These guidelines will not answer every question concerning application of the laws regarding sexual harassment. The Commission issues these guidelines pursuant to its statutory authority to enforce the Commonwealth's anti-discrimination laws. This Commission exists to enforce Mass. Gen. L. ch. 151B and is not bound by federal law. However, "the Federal guidelines can be used to guide Massachusetts in interpreting G.L. c. 151B." LaBonte v. Hutchins & Wheeler, 424 Mass. 813, 823 n.13 (1997).

Important note: The case cites in the footnotes to these guidelines are provided for reference purposes only and in no way limit the Commission's ability to interpret the anti-discrimination laws

2 Mass. Gen. Laws ch. 151B §4(1), (16A) (1996).

3 Mass. Gen. Laws ch. 272, §92A, 98A (1996).

4 Mass. Gen. Laws ch. 151C (1996).

5 Mass. Gen. Laws ch. 151B §4 (1996).

6 Mass. Gen. Laws ch. 151B, §1(5) (1996) (definition of employer).

7 Mass. Gen. Laws ch. 151B§ 1(18)(a) (1996).

8 See, e.g., Hinojosa v. Durkee, 19 MDLR 14, 16 (1997); Mass. Gen. Laws ch. 151B §1(18)(a) (1996).

9 Meritor Sav. Bank, FSB v. Vinson, 106 S. Ct 2399, 2406 (1986); Mass. Gen. Laws ch. 151B §1(18)(a); 42 U.S.C. §2000e et seq.

10 Although these Guidelines use a female pronoun to refer to the "complainant" or the victim of sexual harassment, a victim can be of either sex, and harassment can occur between people of the same sex.

11 See, e.g., Richards v. Walter Fernald State School, 2000 WL 1473024 at *3 (Mass. Super. Ct. July 31, 2000) (where the court denied summary judgment on a quid pro quo claim as a jury could find an encounter in which plaintiff's supervisor said he would "forget all about" the problem of plaintiff not signing a memo if she would "go out" with him amounted to quid pro quo sexual harassment).

12 An employer may also be liable for quid pro quo sexual harassment if an employee loses an opportunity or benefit to another employee because of the other employee's submission to sexual advances or requests for sexual favors. See EEOC Sexual Harassment Guidelines, 29 CFR 1604.11(g).

13 Timko v. City of Boston, 1994 WL 879866, at *12 (Mass. Super. Ct. Aug. 1, 1994).

14 Shanley v. Pub 106, Inc., 22 MDLR 333, 336 (2000).

15 Id. at 336.

16 Bradbury v. The Elbow Room, 18 MDLR 107, 108 (1996). See also Richards, 2000 WL 1473024, at *3 (complainant could base quid pro quo claim on supervisor's request that she "go out" with him).

17 Lipchitz v. Raytheon Co., 434 Mass. 493, 504-05 (2001); Abramian v. President and Fellows of Harvard College, 432 Mass. 107, 116-17 (2000).

18 Abramian, 432 Mass. at 117.

19 Pursuant to Chapter 151B, the analysis governing a hostile work environment claim applies to claims of harassment based on all Chapter 151B protected categories. For example, an employee who is subjected to derogatory or insulting comments or images referencing his or her race may have a claim

for racial harassment. Rainey v. Monsanto Co., 22 MDLR 303, 309-10 (2000); see also Lazure v. Transit Express, Inc., 22 MDLR 16, 18 (2000) ("it is appropriate to analyze a claim of ... racial harassment[] using the models established in sexual harassment cases").

20 Mass. Gen. Laws ch. 151B §1 (18)(b) (1996).

21 Ramsdell v. Western Massachusetts Bus Lines Inc. 415 Mass. 673, 678-79 (1993); Lawless v. Northeast Battery & Alternator, Inc., 22 MDLR 138, 142 (2000).

22 Massachusetts Commission Against Discrimination Model Sexual Harassment Policy 96-2. See, e.g., Cuddyer v. The Stop & Shop Supermarket Co., 434 Mass. 521, 523-26 (2001); Muzzy v. Callihane Motors, Inc., 434 Mass. 409, 410-11 (2001).

23 Melnychenko v. 84 Lumber Co., 424 Mass. 285, 290 (1997). A closely related cause of action not addressed in these guidelines is gender harassment. To establish liability for harassment based on gender, a complainant must establish that (1) she is a member of a protected class; (2) she was the target of speech or conduct based on her membership in that class; (3) the speech or conduct was sufficiently severe or pervasive to alter her conditions of employment and create an abusive working environment; and (4) the harassment was carried out by an employee with a supervisory relationship to complainant, or respondent knew or should have known of the harassment and failed to take prompt remedial action. Fluet v. Harvard University, 23 MDLR 145, 161 (2001); Lazure v. Transit Express, Inc., 22 MDLR 16, 18 (2000). See also Morrison v. Carleton Woolen Mills, Inc., 108 F.3d 429, 441 (1st Cir. 1997) ("[m]any different forms of offensive behavior may be included within the definition of hostile environment sexual harassment") (citing Spain v. Gallegos, 26 F.3d 439, 447 (3d Cir. 1994)). However, the overtones of such behavior must be, at the very least, sex-based, so as to be a recognizable form of sex discrimination").

24 Ramsdell, 415 Mass at 678, n.4; see also Lawless, 22 MDLR at 142-43 (complainant's voluntary participation to some degree in the sexual banter that permeated the workplace did not bar recovery on her sexual harassment claim as complainant used vulgar language because she needed to fit into the environment and minimize the workplace friction).

25 Rosati v. Town of Warren Board of Health, 19 MDLR 34, 38 (1997).

26 Melnychenko, 424 Mass. at 287-88.

27 Kelley v. Plymouth County Sheriff's Department, 22 MDLR 208, 214 (2000).

28 Ramsdell, 415 Mass. at 678-79.

29 Id.

30 Kelley, 22 MDLR at 214 (2000) (quoting Carlton v. Worcester School Dept., 14 MDLR 1143, 1147 (1992)).

31 Ruffino v. State Street Bank and Trust Co., 908 F. Supp. 1019, 1036 n.28 (D. Mass. 1995).

32 Gnerre v. MCAD, 402 Mass. 502, 508-09 (1988). Compare Sweeney v. K-Mart Corp., 21 MDLR 79 (1999) with Dietrich v. Swartz & Corcoran Ins. Agency, Inc., 22 MDLR 367 (2000).

33 Kelley, 22 MDLR at 214- 15.

34 Id. at 215.

35 Prader v. Leading Edge Products, Inc., 39 Mass. App. Ct. 616, 619-20 (1996).

36 College-Town Division of Interco, Inc., v. MCAD, 400 Mass. 156, 162 (1987); Mass. Gen. Laws ch. 151B §1(18)(b) (1996).

37 Ramsdell, 415 Mass. at 678.

38 McAuliffe v. Suffolk County Sheriff's Dept., 21 MDLR 27, 29 (1999).

39 Richards v. Bull HN Information Systems, Inc., 16 MDLR 1639, 1666 (1994).

40 See Smith-Pereira v. Coffees of the World, 20 MDLR 229, 234 (1998).

41 Melnychenko, 424 Mass. at 290.

42 See id., citing Smith v. Brimfield Precision, Inc., 17 MDLR 1089, 1096 (1995). See also Mass. Gen. Laws ch. 151B, §1(18) (1996).

43 Melnychenko, 424 Mass. at 290.

44 Johnson v. Boston Edison Co, 19 MDLR 162, 166 (1997); Blades v. Hahjee's Place, 7 MDLR 1741, 1752-54 (1985).

45 Id. See also Borrelli v. Aria Technologies, 21 MDLR 103, 105 (1999) (citing Sanderson v. Town of Wellfleet, 16 MDLR 1341 (1994).

46 Borrelli, 21 MDLR at 105-06.

47 Johnson v. Boston Edison Co., 19 MDLR at 166-67.

48 College-Town, 400 Mass. at 165-167; Holt v. Minuteman Flames Minor Hockey Ass'n, 22 MDLR 373, 375 (2000); Miller v. Berkshire Hospitality Corp., 22 MDLR 321, 324 (2000); Rowe v. American Paper Products, Inc., 22 MDLR 279, 282 (2000); Messina v. Araserve, Inc., 906 F. Supp. 34, 37 (D. Mass. 1995); LeClerc v. Interstate Distrib. Div. of Hudson News Co., 2000 WL 33170694 at *6 (Mass. Super. Ct. Aug. 31, 2000).

49 College-Town, 400 Mass. at 165-67.

50 Id.

51 See, e.g., LeClerc, 2000 WL 33170694, at *6.

52 See, e.g., Fluet v. Harvard University, 23 MDLR 145, 163 (2001); Doucimo v. S & S Corp., 22 MDLR 82, 87 (2000); Albee v. New England Medical Center Hosp., Inc., 1997 WL 691524, at *7 (Mass. Super. Ct. Oct. 30, 1997); Lazure v. Transit Express, Inc., 22 MDLR 16, 19 (2000); Levesque v. Quality Brands Liquor, 19 MDLR 154, 156 (1997); Przybycien v. Aid Maintenance Co., 13 MDLR 1267, 1281 (1991). See also Enforcement Guidance on Vicarious Liability for Unlawful Harassment by Supervisors, EEOC Notice No. 915.002, June 18, 1999, subsection III ("Who Qualifies as a Supervisor?") (A copy of this EEOC Guidance can be obtained at http://www.eeoc.gov/docs/harassment.html).

53 Under Massachusetts law, apparent authority is created when "written or spoken words or any other conduct of the principal which, reasonably interpreted, causes a third person to believe that the principal consents to have the act done on his behalf by the person purporting to act for him." Zortman v. Bildman, 1999 WL 1318959, at *11 (Mass. Super. Ct. Jan. 15, 1999) quoting Weisman v. Saetz, 11 Mass. App. Ct. 440, 442 (1981); see also Restatement (Second) of Agency §§ 7, 8 (1957) ("Apparent authority is the power to affect the legal relations of another person by transactions with third persons,

professedly as agent for the other, arising from and in accordance with the other's manifestations to such third persons.").

54 See, e.g., Przybycien, 13 MDLR at 1281; Robinson v. Haffner's Service Stations, Inc., 23 MDLR 283, 286-87 (2001).

55 Robinson, 23 MDLR at 286- 87; Weisman v. Saetz, 11 Mass. App. Ct. 440, 442 (1981); Restatement (Second) of Agency § 8, comment c (1957); Galotti v. Town of Belmont, 6 MDLR 1669, 1687-88 (1984).

56 College-Town, 400 Mass. at 165-66; Comeau v. Idea Lube, Inc., 22 MDLR 5, 7 (2000); Levesque, 19 MDLR at 156; Messina v. Araserve, Inc., 906 F. Supp. 34, 38 (1995); Battenfield v. Harvard University, 1993 WL 818920 at *4 (Mass. Super. Ct. Aug. 31, 1993).

57 Actual knowledge is most commonly established by demonstrating that the employee informed a supervisory level employee of the alleged harassment. See, e.g., Jordan v. Prime Laminating, Inc., 22 MDLR 151, 153 (2000).

58 The employer may be held liable when it is on constructive notice of unlawful conduct, or when it "should have known" of the conduct. See, e.g., Parent v. Spectro Coating Corp., 22 MDLR 221, 225-26 (2000). Cf. Gilman v. Instructional Systems, Inc., 22 MDLR 237, 241 (the complainant failed to prove that respondent "knew or should have known" about harassment allegedly perpetrated by her co-worker because she never informed her supervisor and, the only person she did inform was another co-worker).

59 See, e.g., Beldo v. Univ. of Massachusetts Boston, 20 MDLR 105, 111 (1998); Richards, 16 MDLR at 1668-69; Parent, 22 MDLR at 225-6; Rounds v. Dep't. of Corrections, 19 MDLR 90, 100 (1997); Russell v. Hillcrest Educational Centers, Inc., 23 MDLR 91, 95-96 (2001).

60 Rodriguez-Hernandez v. Miranda- Velez, 132 F.3d 848, 854 (1st Cir. 1998) (customer has no liability, but employer liable for discharging plaintiff after she complained of sexual harassment by customer); Burman v. Boch Oldsmobile, Inc., 1995 WL 809940 at *4 (Mass. Super. Ct. Apr. 11, 1995). See, e.g., Folkerson v. Circus Enterprises, Inc. 107 F.3d 754, 756 (9th Cir. 1997); Powell v. Las Vegas Hilton Corp., 841 F. Supp. 1024, 1028 (D. Nev. 1992).

61 Ligenza v. Genesis Health Ventures of Massachusetts, Inc., 995 F. Supp. 226, 230-31 (D. Mass. 1998).

62 See, e.g., Handy v. North End Community Health Center, Inc., 21 MDLR 37, 38 (1999).

63 Fiske v. R.P. Liquor, Inc., 16 MDLR 1042, 1049-50 (1994) (company liable for hostile work environment caused by a friend of the owner).

64 Handy, 21 MDLR at 38 (Commission concluded that respondent was not liable for the conduct of a client because it had insufficient control over client and took measured and reasonable steps to alleviate the harassing conduct).

65 Beaupre v. Cliff Smith & Associates, 50 Mass. App. Ct. 480, 490-91 (2000) (employer's president individually liable for both quid pro quo and hostile work environment sexual harassment; plain language of the statute provides on its face for individual personal liability for unlawful sexual harassment actively perpetrated by president himself); Ruffino v. State Street Bank and Trust Co., 908 F. Supp. 1019, 1048 (D. Mass. 1995).

66 Raffurty v. Keyland Corp., 22 MDLR 125, 127 (2000) (president and owner of employer corporation who made such persistent verbal and physical sexual overtures to employee that she was finally

compelled to quit held personally liable for hostile work environment sexual harassment; corporation also vicariously liable).

67 MCAD v. Medical Weight Loss Center, Inc., 23 MDLR 5, 12 (2001) (owner held individually liable where he coerced, intimidated, interfered with, and retaliated against the complainant in the exercise of her right to be free from harassment based on her sex and sexual orientation).

68 Compare Deeter v. Bravo's Pizzeria and Restaurant, 23 MDLR 167 (2001) with Rushford v. Bravo's Pizzeria and Restaurant, 23 MDLR 171 (2001); see also Doucimo v. S & S Corp., 22 MDLR 81, 87 (2000); Morehouse v. Berkshire Gas Co., 989 F. Supp. 54, 61 (D. Mass. 1997). An individual person can also be liable for acts of retaliation under § 4(4). See Bain v. City of Springfield, 424 Mass. 758, 765 (1997) (mayor's directive to "get rid of" plaintiff after she complained about discriminatory behavior was retaliatory and mayor could be held individually liable); Kelley, 22 MDLR at 217 (complainant won damage award against department and individuals for unlawful retaliation because the individuals had participated in, were aware of, or could have prevented the retaliatory conduct). This same rationale applies in the context of other prohibited forms of discrimination. Bendell v. Lemax, Inc., 22 MDLR 259, 263 (2000) (company president held individually liable under ch. 151B, § 4(4A) for interfering with complainant's right to be free from unlawful discrimination when he terminated her because of her disability).

69 Rosati v. Town of Warren Bd. of Health, 19 MDLR 34, 38 (1997); Carney v. Town of Falmouth Police Department, 16 MDLR 1444, 1455-56 (1994), aff'd 17 MDLR 1280 (1995) (Full Comm'n); Morehouse, 989 F. Supp. at 63-64.

70 Tunstall v. Acticell H'W Cosmetics, 22 MDLR 284, 287-88 (2000).

71 Erewa v. Reis, 20 MDLR 36, 38 (1998).

72 See Harmon v. Malden Hosp., 19 MDLR 157, 157-58 (1997), See also Murray v. Sharp Air Freight Services, Inc., 2000 WL 33170935, at *4 (Mass. Super. Ct. Dec. 5, 2000); Hope v. San Ran, Inc., 8 MDLR 1195, 1211, aff'd, 8 MDLR 1277 (1986). Cf. Hennessy v. Perico, Inc., 1999 WL 515078 at *1 (Mass. Super. Ct. May 20, 1999).

73 See Murray, 2000 WL 33170935, at *4 (citing Harmon).

74 Harmon, 19 MDLR at 158; Chapin v. University of Massachusetts at Lowell, 977 F. Supp. 72, 80 (D. Mass. 1997).

75 LeClerc v. Interstate Distrib. Div. of Hudson News Co., 2000 WL 33170694 at *10 (Mass. Super. Ct. Aug. 31, 2000), citing Chapin, 977 F.Supp. at 77-80.

76 Morehouse, 989 F. Supp. at 61-62.

77 See, e.g., DeBarboza v. Cablevision of Boston, Inc., 1999 WL 65489, at *2 (Mass. Super. Ct. Jan. 29, 1999) (although court held that corporate defendant could be held liable as an aider and abettor, it dismissed the claim because the plaintiff failed to allege the person or persons that the corporation allegedly aided or abetted).

78 Beaupre, 50 Mass. App. Ct. at 495 n.23 (noting that the actor must be found to have the requisite intent to discriminate in order to be liable for aiding and abetting); Urrea, slip op. at 6, supra note 57; Harmon, 19 MDLR at 158. Compare Russell v. Hillcrest Educational Centers, Inc., 23 MDLR 91, 96 (2001) (aiding and abetting claim dismissed due to lack of evidence demonstrating respondent's agents

had knowledge of the harasser's conduct and intended to assist the harasser in his alleged unlawful actions).

79 See, e.g., Beaupre, 50 Mass. App. Ct. at 494-95 (president and controlling shareholder of respondent company held liable as an aider and abettor for acting jointly with the respondent company in both his separate acts of sexual harassment against the complainant, which the company is deemed vicariously to have authorized him to do, and in causing the company to sanction the complainant's demotion and subsequent constructive discharge), citing Model Penal Code and Commentaries § 2.06(2) (c), (3)(a) and comments 5-6; § 2.07(6) (a) and comment 7 (1980) (which makes a person "legally accountable for any conduct he performs or causes to be performed in the name of the corporation or in its behalf to the same extent as if it were performed in his own name or behalf" and "makes certain that the corporate agent will not escape liability because all or part of his conduct is performed through or in the name of the corporation").

80 Mass. Gen. Laws ch.151B §3A (1996). As defined in §1 of the statute, covered employers are those companies that employ six or more employees. See Mass. Gen. Laws ch. 151B, §1(5) (1996).

81 Mass.Gen.Laws ch. 151B §3A(b) (1996).

82 Mass. Gen. Laws ch. 151B, §3A(b)(2) & (c) (1996). A copy of the MCAD Model Sexual Harassment Policy No. 96-2 and the poster can be obtained through the MCAD. The Model Policy is accessible online at http://www.state.ma.us/mcad/harassment.html. The Commission further recommends that employers establish an anti-harassment policy that clearly prohibits harassment based on all classes protected by the statute.

83 Distribution of pornography may be part of the harassment. Bowman v. Heller, aff'd in part and vacated in part by 420 Mass. 577 (1995), 1993 WL 761159 at *7 (Mass. Super. Ct. Jul. 9, 1993), Barbetta v. Chemlawn Services Corp., 669 F. Supp. 569, 572 (W.D.N.Y. 1987).

84 Mass. Gen. Laws ch. 151B, §3A(b)(2) (1996). In addition, it is recommended that the employer post both the policy and the poster in a conspicuous area in the workplace.

85 Mass. Gen. Laws ch. 151B, §3A(e) provides that employers are encouraged to conduct an education and training program for new employees within one year of commencement of employment. See also Charles Walker, "An Ounce of Prevention...," MCAD Certified Discrimination Prevention Training Program, Sexual and Other Harassment Prevention Module, MCLE (2000-15-CM). In 1999, the Commission, in conjunction with the MCLE, initiated a program to train individuals to educate workforces on techniques to prevent discrimination in the workplace. A list of sexual harassment prevention trainers is available from the Commission.

86 Mass. Gen. Laws ch. 151B, §3A(e) (1996). The training should make it clear that harassment based on the other protected categories violates Chapter 151B.

87 Id. The statute provides that the additional training should be commenced within one year of employment for new managers and supervisors.

88 See Section III, text at 8 - 10.

89 See Harley v. Costco Wholesale Corp., 21 MDLR 87, 93-94 (1999) (employer's failure to take prompt remedial action was supported by evidence that respondent failed to adequately train its managers and supervisors in how to respond to a complaint of discrimination or harassment, rendering them ineffective in this regard). In court cases where punitive damages may be sought, evidence of training may also mitigate damages.

90 See Rounds v. Dep't. of Corrections, 19 MDLR 90, 99-100 (1997) (finding of no liability on the part of the employer which promptly assigned an "experienced special investigator" to meet with witnesses to the alleged harassing conversation).

91 It is important to have options with respect to how the employee may report a complaint of harassment, as the complaint may involve the individual whose position it is to handle sexual harassment complaints.

92 Parent, 22 MDLR at 226.

93 See Beldo, 20 MDLR at 107, 112 (investigation that involved one interview between the investigating supervisor and the alleged perpetrator - both of whom were friends - and no one else, not even the complainant, held to be evidence of an inadequate investigation).

94 See, e.g., Johnson v. Boston Edison Co., 19 MDLR at 165 (investigation appropriately included review of internal company telephone records).

95 See, e.g., Michel v. Factory Mut. Engineering Corp., 1998 WL 726500, at *9 (Mass. Super. Ct. Oct. 2, 1998) (employer promptly painted over bathroom graffiti).

96 See, e.g., Rainey v. Monsanto Co., 22 MDLR 303, 310 (2000) (employer not only took photographs of offending graffiti and painted the walls, but also increased maintenance and supervisory surveillance of the restrooms, had supervisory and management meetings with all work crew to discuss incident and to re-convey management's anti-harassment policy, and followed up with diversity consultants and continued investigations, involving the entire workforce of the division).

97 It is important to note that the employer's determination as to whether sexual harassment did or did not occur does not in any way bind the Commission to make the same finding.

98 EEOC Policy Guidance on Sexual Harassment, N-915-050, 8 FEP Manual No. 6699, (March 19, 1990) (hereinafter, "EEOC Policy Guidance on Sexual Harassment"); Harley, 21 MDLR at 93.

99 See, e.g., Michel, 1998 WL 726500, at *9.

100 "EEOC Policy Guidance on Sexual Harassment."

101 See, e.g., Sarin v. Raytheon Co., 905 F. Supp. 49, 53 (D. Mass. 1995); cf. Hoffman v. Daka, Inc., 2000 WL 33170817, at *4 (Mass. Super. Ct. Oct. 31, 2000).

102 See, e.g., Johnson v. Boston Edison Co., 19 MDLR at 16. See "EEOC Policy Guidance on Sexual Harassment." See also Harley, 21 MDLR at 93; Barbot v. Hapco Farms, Inc., 19 MDLR 133, 137 (1997).

103 On August 7, 2002, Governor Swift signed into law chapter 223 of the Acts of 2002, which amended Mass.Gen.Laws. c.151B, §5 to change the statute of limitations from six months to 300 days. The amendment goes into effect on November 5, 2002 and applies to claims arising after that date. For all claims arising before that date, the six-month limitations period still applies.

104 Ruffin., 908 F. Supp. at 1037.

105 See, e.g., Conroy v. Boston Edison Co., 758 F. Supp. 54, 60-61 (D. Mass. 1991), Andrews v. Arkwright Mutual Insurance Co., 423 Mass. 1021, 1022 (1996); Adams v. Mount St. Vincent Nursing Home, 19 MDLR 123, 125 (1997).

106 Wheatley v. American Tel. & Tel. Co., 418 Mass. 394, 398 (1994) (citing McConnell v. General Tel Co. of Cal., 814 F.2d 1311, 1317 (9th Cir. 1987); Wynn & Wynn v. MCAD, 431 Mass. 655, 673

(2000); DeBiase v. MBTA, 22 MDLR 271, 273-274 (2000); Soriano v. City of Lawrence Police Department, 2000 WL 1584852 at *3 (Essex Super. Ct. Oct. 23, 2000).

107 MCAD Rules and Procedures 804 CMR 1.10(2) (2002) (emphasis added); Cuddyer v. Stop & Shop Supermarket Co., 434 Mass. 521 (2001).

108 Cuddyer, 434 Mass. at 532-33.

109 Id. at 532.

110 Id. at 532 (citations omitted). See also Ruffino, 908 F. Supp. at 1038-39; Olsen v. Lenco Industries, Inc., 19 MDLR 176, 181 (1997).

111 Mortimer v. Atlas Distr. Co., 15 MDLR 1233, 1250 (1993).

112 Carter v. Commissioner of Correction, 43 Mass. App. Ct. 212, 220-21 (1997). Cf Clifton v. MBTA, 2000 WL 218397 at *6 (Feb. 3, 2000); Beldo v. Univ. of Massachusetts at Boston, 20 MDLR 105, 110-111 (1998); Lynn Teachers Union, Local 1037 v. MCAD, 406 Mass. 515, 523 (1990). In contrast, where a continuing violation is not established, the complainant is limited in using the untimely events as background evidence to establish a hostile work environment, and she may not recover damages for the time-barred events. Cuddyer, 434 Mass. at 530, n.10.

113 Cuddyer, 434 Mass. at 532-33.

114 De Almeida v. The Children's Museum, 2000 WL 96497, at *3 (Suffolk Super. Ct. Jan. 11, 2000) (A sexual harassment claim must be more than the "residual effects of past discrimination.").

115 Cuddyer, 434 Mass. at 533.

116 Id. at 531, n.12.

117 See Goguen v. Quality Plan Administrators, 2000 WL 282485 at *3 (Mass. Super. Ct. Feb. 1, 2000).

118 See De Almeida,2000 WL 96497, at *3 (hateful looks, the only alleged conduct within the six-month period, were not found to be "substantially related" to the earlier allegations of harassment).

119 Cuddyer, 434 Mass. at 539. The SJC has stated that this test is more favorable to the complainant and differs substantively from the federal "reveletory" standard. That standard provides that once a complainant knows, or reasonably should know, that she is being sexually harassed, the limitations clock begins to run. Id. at 537-38.

120 Cuddyer, 434 Mass. at 530 n.10 (2001).

121 Jensen v. Frank, 912 F.2d 517, 523 (1st Cir. 1990). See, e.g., Lynn Teachers Union, Local 1037, 406 Mass. at 522-523 (approving application of continuing violation rule where refusal to credit complainants for pre- resignation seniority breathed new life into discriminatory maternity leave policy).

122 GTE Products Corp. v. Stewart, 421 Mass. 22, 34 (1995), citing Alicea Rosado v. Garcia Santiago, 562 F. 2d 114, 119 (1st Cir. 1977); Holt at 375; Choukas v. Ocean Kai Restaurant, 19 MDLR 169, 171 (1997); Said v. Northeast Security, 18 MDLR 255, 259 (1996).

123 See Parent, 22 MDLR at 226 (co-worker's lewd gestures, physical and verbal threats, unwelcome touching, and phone calls combined to meet criteria for constructive discharge).

124 Langford v. Dep't of Employment and Training, 17 MDLR 1043, 1063 (1995), aff'd, 18 MDLR 36 (1996) (Full Comm'n).

125 GTE Products, 421 Mass. at 35, citing Stetson v. NYNEX Serv. Co., 995 F.2d 355, 361 (1st Cir. 1993); Smith-Somers v. Fathers and Sons Pizza, Inc., 19 MDLR 1, 2 (1997) aff'd 24 MDLR 88 (2002).

126 See GTE Products, 421 Mass. at 36 (employee's obligation to act reasonably includes "an obligation not to assume the worst and not to jump to conclusions too fast") (quoting Garner v. Wal-Mart Stores, Inc., 807 F.2d 1536, 1539 (11th Cir. 1987); Bengin v. Braintree Highlands Corp., 21 MDLR 197, 199 (1999); Kletkotka v. Carlin Combustion Technology, Inc., 21 MDLR 72, 75 (1999); Estate of Douglas McKinley v. Boston Harbor Hotel, 14 MDLR 1226, 1241 (1992).

127 Parent, 22 MDLR at 226 (respondent's failure to take prompt and effective action to remedy harassment led to constructive discharge).

128 For more information about investigating claims of sexual harassment and taking effective remedial action to eliminate harassment, see Section VI of these Guidelines

129 This provision protects both current and former employees. Robinson v. Shell Oil Co., 117 S. Ct. 843, 848-49 (1997) (interpreting Title VII).

130 Tate v. Dep't of Mental Health, 419 Mass. 356, 364-65 (1995); MacCormack v. Boston Edison Co., 423 Mass. 652, 662-6 (1996).

131 Abramian, 432 Mass. at 121.

132 Bain, 424 Mass. at 765.

133 See Logan v. Brook Farm Egg, No. 97-BEM-1899, 24 MDLR ____ (2002).

134 MacCormack, 423 Mass. at 663.

135 Kealy v. City of Lowell, 21 MDLR 19, 24 (1999).

136 Farricy v. Suffolk County Sheriff's Dep't, 22 MDLR 27, 28-29 (2000) (Full Comm'n).

137 Pulido v. Dep't of Corrections, 20 MDLR 49, 52 (1998).

138 Muise v. Credit Exchange, 17 MDLR 1684, 1691-92 (1995). In fact, as described in Section VII(B) of these Guidelines, such retaliation can constitute a part of a continuing violation that relates back and includes otherwise untimely incidents of sexual harassment.

139 Monahan v. Dep't of Mental Retardation, 14 MDLR 1049, 1053 (1991), aff'd, 14 MDLR 1153 (1992) (Full Comm'n). See also Bass v. Dep't of Mental Retardation, 20 MDLR 1, 4 (1998) (retaliation claim dismissed where employer fired complainant prior to being served with a copy of her MCAD charge and employer had no other reason to know about charge).

140 Tate, 419 Mass. at 364. Causation may be inferred from proof of retaliatory intent. Complainants frequently use the three stage burden shifting model of Blare v. Husky Injection Systems, Boston, Inc., to prove retaliatory intent. Under this analysis, protected activity, adverse action, and causal nexus/proximity in time are the elements of the prima facie case, followed by the rebuttal and pretext stages. 419 Mass. 437, 441-443 (1995); see also Ruffino, 908 F. Supp. at 1044-47. A fact finder can then infer causation from proof of retaliatory intent.

141 See Hurd v. Massachusetts Port Authority, 20 MDLR 11, 19 (1998); Cimino v. BLH Electronics, Inc., 5 MDLR 1286-87, 1285 (1983); Emmons v. Codex Corp., 4 MDLR 1523, 1551 (1982).

142 See, e.g., Socarides v. Camp Edwards Troop Welfare Council, Inc., 21 MDLR 173, 177 (1999).

143 See Roy v. New England Steak House, Inc., 23 MDLR 299, 303-04 (2001).

144 Riggs v. Town of Oak Bluffs, 23 MDLR 306, 311 (2001). See also Bain, 424 Mass. at 765 n.4; Melnychenko 424 Mass. at 294-295 (§ 4(4A) may extend to use of termination agreements and/or waivers which interfere with employee rights under M.G.L. ch. 151B); Harmon, 19 MDLR at 157 (for guidance in interpreting § 4(4A), MCAD looks to cases under Massachusetts Civil Rights Act, Mass. Gen. Laws ch. 12, § 11H).

145 Mass. Gen. Laws ch. 151B §8 (1996).

EXHIBIT G

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Boston Children's Hospital

About Us

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Respect
Inclusivity
Teamwork
Kindness

It's the Boston Children's Way

At Boston Children's, we hold ourselves to the highest values of respect, inclusivity, teamwork, and kindness to provide patients, families, and each other with an experience equal or better than the care we deliver.

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clinical care, biomedical research, medical education and community engagement.

Boston Children's is ranked among the best in the nation by *U.S. News and World Report*. It is home to the world's largest pediatric research enterprise, and it is the leading recipient of pediatric research funding from the National Institutes of Health. It is the primary pediatric teaching hospital for Harvard Medical School. Boston Children's treats more children with rare diseases and complex conditions than any other hospital.

Vision and mission

For over 150 years, Boston Children's Hospital has maintained the same vision: to advance pediatric care worldwide. Our four-part mission is to:

- provide the highest quality of health care
- lead the way in research and discovery
- educate the next generation of leaders in health care
- enhance the health and well-being of the children and families in our local community

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Boston Children's is dedicated to creating a culture where all patients, families, clinicians, researchers, staff, and communities feel empowered and supported. We are committed to working together to support health equity and promote anti-racist practices. This is not merely an aspirational goal, and in August 2020, we shared our formal Declaration on Equity, Diversity and Inclusivity that will be our guiding compass in making this goal a reality.

- Learn more about the Office of Health Equity and Inclusion.
- Learn more about our commitment to LGBTQ Equality.
- Learn more about our commitment to Supplier Diversity.

Collecting Race, Ethnicity, Language, and Education Level from Patients and Families

At Boston Children's Hospital, we are committed to providing our patients the best health care. Getting to know the background and preferences of our patients helps us deliver high quality care.

- + Why are you collecting this information?
- + Who can see this information?
- + What will you do with this information?
- + What if I don't want to share my child's information with you?
- + Is this an immigration/citizenship question?
- + Can't you tell by looking at me or my child?
- + What if I am more than one race?
- + What should I put if I'm an American?
- + Are other hospitals collecting this?
- + How are race and ethnicity related to my child's health?

Our capacity

With more than 40 clinical departments and 258 specialized clinical programs, Boston Children's is one of the largest pediatric medical centers in the United States. We provide a complete range of health care services for children of all ages, and in some cases, we can offer fetal interventions and treatments for adults.

Learn more about our extensive expansion and renewal projects that will increase capacity and ensure the highest quality care for patients and families.

Locations

Whether you're living in Boston, the suburbs or somewhere else in the Northeast, the world-class care of Boston Children's Hospital is never far away. Boston Children's is ranked among the best in the nation by *U.S. News & World Report*. Families are always our #1 partners in care.

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4.7	4.8	4.8
★★★★½	★★★★½	★★★★½
Ambulatory Clinic	**Day Surgery**	**Inpatient**
Based on 21,708 surveys	Based on 1,783 surveys	Based on 1,123 surveys

Boston Children's Hospital is committed to better understanding and improving the experiences for our patients and families. After a visit to Boston Children's, you may receive a survey by mail, email, or text message from Press Ganey, a nationally recognized survey vendor. Your feedback is important to us. Responses from the surveys help us improve and provide the best care to all patients and families.

The star ratings displayed are the average of responses collected in our Patient Experience Surveys. This rating averages the scores for the 'Would you recommend Boston Children's to your friends and family' question, displayed on a scale of 1 to 5, with 5 indicating the best experience.

Our leadership

"The commitment and compassion with which we care for all children and families is matched only by the pioneering spirit of discovery and innovation that drives us to think differently, to find answers, and to build a better tomorrow for children everywhere."

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Our partnerships

We are proud to be the primary teaching hospital of Harvard Medical School, and our Nursing Department partners with 27 schools of nursing throughout Massachusetts and New England.

Dana-Farber/Boston Children's Cancer and Blood Disorders Center, an integrated pediatric hematology and oncology program through Dana-Farber Cancer Institute and Boston Children's Hospital, provides – in one specialized program. We also maintain close working relationships with Brigham and Women's Hospital, Massachusetts General Hospital and many other are hospitals in caring for our patients.

Boston Children's Hospital Employee Medical Plan Price Transparency

This link leads to the machine-readable files that are made available in response to the federal Transparency in Coverage Rule and includes negotiated service rates and out-of-network allowed amounts between health plans and healthcare providers. The machine-readable files are formatted to allow researchers, regulators, and application developers to more easily access and analyze data. It is not intended to be a cost comparison tool for employees and dependents in Boston Children's Hospital's medical plans. To access BCBSMA's plan cost comparison tools, please visit https://Transparency-in-Coverage.Bluecrossma.com.

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EXHIBIT H

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About Us

Health Equity

For more than 150 years, Boston Children's has embodied a culture of innovation and scientific discovery, dedicated to improving the lives of children and families. We have taken pride in providing a safe and welcoming environment for those who come to us for care and every member of our team.

Yet racism, discrimination, and hate continue to harm many of members of the Boston Children's community physically, mentally, and emotionally. Further, the COVID-19 pandemic and its disproportionate toll on underrepresented communities has made health disparities that undermine children's health even more apparent.

Boston Children's is unwavering in its commitment to ensuring every child has access to the care they need and deserve. To remain true to this commitment, the hospital has implemented the following ongoing initiatives.

Boston Children's Declaration on Equity, Diversity, and Inclusivity

In 2020, as part of a national reckoning with racial injustice and health disparities, Boston Children's released a Declaration on Equity, Diversity, and Inclusivity. The declaration established six goals to elevate health equity as an enterprise priority, core to everything we do.

As we plan and strategize for the future, these goals serve as a roadmap in our work to recognize the impact of racism on children's health, promote optimal health for our diverse patients, and ensure Boston Children's is a welcoming place for all who seek our care as well as every person who works here.

- **Goal 1**: Boston Children's is committed to being an inclusive environment that does not tolerate any form of racism, discrimination, or bias.
- **Goal 2**: Boston Children's is committed to recruiting, developing, and retaining a diverse workforce.
- **Goal 3**: Boston Children's is committed to eliminating structural racism from all policies, guidelines, and practices.
- **Goal 4**: Boston Children's is committed to comprehensively educating employees on the impact of racism on child health, unconscious bias, bystander-upstander awareness, and the role of difficult conversations in culturally effective pediatric care delivery.
- **Goal 5**: Boston Children's is committed to being a leader in eliminating child health disparities in our community and our nation.
- **Goal 6**: Boston Children's Hospital is committed to leading in the development, implementation, and tracking of metrics for equity, diversity, and inclusion.

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The Office of Health Equity and Inclusion works with departments and divisions throughout Boston Children's to support health equity and promote culturally effective care. Among our many initiatives, we work to recruit, develop, and advance a diverse, inclusive workforce. Our enterprise-wide training on standing up to microaggressions helps ensure every Boston Children's patient and staff feels welcome and respected here.

Sandra L. Fenwick Institute for Pediatric Health Equity and Inclusion

The Sandra L. Fenwick Institute for Pediatric Health Equity and Inclusion advances pediatric health equity locally, nationally, and around the world. The institute carries out this work in three primary areas: health equity research, inclusion, and public policy.

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